CHAPTER 4

Theories and Techniques of Group Counseling

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CHAPTER LEARNING OBJECTIVES

1. Provide a general framework for theory applied to practice
2. Understand how techniques are associated with each theory of group work
3. Consider ways of viewing a group through a multicultural lens
4. Provide an overview of the theoretical foundations of group counseling and group work (CACREP, 2016, Standard A)
5. Describe the key concepts and techniques of psychoanalytic approaches to group work
6. Describe the key concepts and techniques of experiential and relationship-oriented approaches to group work
7. Describe the key concepts and techniques of cognitive behavioral approaches to group work
8. Describe the key concepts and techniques of postmodern approaches to group work
9. Delineate practical ways to develop an integrative approach to group counseling

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It is the first week for you at your internship at a residential treatment center. You were just informed by your supervisor that next week you will be starting a dialectical behavior therapy (DBT) group for six to eight women struggling with borderline personality disorder.

- What are your immediate reactions to leading this type of group?
- How would you approach conducting this group if DBT is (or is not) your theoretical orientation to counseling?
- What theories do you find yourself most drawn to and why?
- How do the theories you use fit with your personality?
- How might your own cultural background, identity, and life experiences influence the theories from which you draw?

Introduction

In this chapter we provide a brief introduction to some of the major theories of counseling that have applicability to group counseling. At the end of each of the theory sections, we list selected readings that we have found particularly useful. To formulate your own perspective on how to apply theory to the practice of group work, you will need to do extensive reading on each of the theories that interest you and the techniques that you might find useful. Our main purpose here is to present a framework for thinking about how to apply techniques in groups and to show the connection between theory and technique.

You may ask why a theory is important and why we devote an entire chapter to this subject. A theory provides you with a structure for designing appropriate interventions and evaluating the outcomes of the group as a whole and of the individual members. This is a way of organizing information you get from the multiple interactions within a group. We think that it is particularly important that you develop a personal stance toward the practice of group counseling that fits the person you are and is flexible enough to meet the unique needs of the members that make up the group.

A theory informs the way you operate in facilitating a group, guiding the way you are with the members and defining both your role and the members' roles in a group. A theory provides a frame of reference for understanding and evaluating the world of the client, especially when it comes to building rapport, making an assessment, defining problems, and selecting appropriate techniques in meeting the goals of the members. Attempting to make interventions and implement techniques as a group leader without having a clear theoretical rationale is much like trying to build a house without a set of blueprints. If you are unable to draw on theory to support your interventions, your groups may not achieve the maximum benefit.

Theory is not a rigid set of structures that prescribes, step by step, what and how you should function as a leader. Rather, theory is a general framework that helps you make sense of the many facets of group process, providing a sense of direction for what you do and say in a group. Theory helps you think about the possible results of your interventions. Being able to articulate your theoretical stance is beneficial for clients as well because it can help to define the roles and expectations of group members.

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Theory as a Roadmap

LO1

A theory can guide you, much like a road map or a navigational instrument. A set of directions tells you where you are starting out, where you hope to end, and the steps along the way. If you do not have a navigational instrument, you may get lost and lose time trying to find your way. However, even with a directional guide you will encounter detours and unforeseen circumstances. Although you have a plan, you will need to be flexible in modifying the way you try to reach your goals. In this chapter we describe our own integrative approach, emphasizing the role of thinking, feeling, and acting in human behavior, which is based on selected ideas from most of the theories presented here.

There are many different theoretical approaches to understanding how the group process works and how change occurs. Different group practitioners work in a variety of ways as they deal with the same themes that emerge in a group, largely based on their theoretical orientation. Some leaders focus on feelings, believing that what members need most is to identify and express feelings that have been repressed. Other leaders emphasize gaining insight and increased self-awareness. For some leaders the emphasis is on practicing new behavior in a group and assisting members in developing specific action plans to bring about desired behavior changes. Others encourage group members to examine their beliefs about themselves and about their world; these leaders believe that change will result if the participants can eliminate inaccurate thinking and replace it with constructive thoughts and self-talk. Each of these choices represents a particular theoretical orientation, and the various theoretical approaches can work in concert to enhance your understanding of how groups work.

Group counselors may give primary attention to these time frames: the past, the present, or the future. It is important to consider whether you see the past, present, or future as being the most productive focus for group work. If you believe your members' past is a crucial aspect to explore, many of your interventions may be aimed at assisting them in understanding how their past is connected to their present behavior. If you think your members' goals and strivings are important, your interventions are likely to focus on the future. If you are oriented toward the present, many of your interventions are likely to emphasize what the members are thinking, feeling, and doing in the moment. With this present-centered orientation, you will probably ask members to bring both past events and future concerns into the here and now.

Our Theoretical Orientation

We are sometimes asked to identify what theory we follow. None of us subscribe to any single theory in its totality. Rather, we function within an integrative framework that we continue to develop and modify as we practice. We draw upon concepts and techniques from most of the contemporary therapeutic models and adapt them to our own unique personalities, our therapeutic styles, and to the needs of the members of a particular group. We operate on the assumption that who we are

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as a person is as important as our knowledge of counseling theory and the level of our skills. Certainly skills and techniques are critical. The techniques we employ can be practiced and refined, and our level of skill development enhances our practice. Although it is essential to use skills and techniques effectively—and to have a theoretical base from which to draw a range of techniques—this ability does not constitute the entire picture. The quality of the therapeutic relationship we establish with members is critical in determining successful outcomes. The methods we use and our theory of choice are important variables, but the relational factors are the foundation of effective group work. We use our skills to enhance our ability to create a sense of connection with the members of our groups. Building upon this relational foundation, our conceptual framework takes into account the *thinking*, *feeling*, and *behaving* dimensions of human experience.

An Integrative Approach to Group Practice We suggest that you study all the contemporary theories to determine what concepts and techniques you most want to incorporate in your leadership style. Developing your counseling theory is a process that will likely take years and will evolve as you gain experience and fine tune your own clinical style. In developing and conceptualizing your integrative approach to counseling, consider your own personality, interpersonal strengths, life experiences, and worldview as you choose the concepts and techniques that work best with a range of clients. To make effective choices, you need to be well grounded in a number of theories. Remain open to the idea that some aspects of these theories can be unified in some ways, and test your hypotheses to determine how well they are working. Getting regular feedback from the members of your groups is one of the best ways to assess how well your interventions are working.

An integrative approach involves the process of selecting concepts and methods from a variety of systems, and there are multiple pathways to achieving this integration. Two of the most common are technical integration and theoretical integration. **Technical integration** tends to focus on differences, uses techniques drawn from many approaches, and is based on a systematic selection of techniques. This path combines techniques from different schools without necessarily subscribing to the theoretical positions that spawned them. In contrast, **theoretical integration** refers to a conceptual or theoretical creation beyond a mere blending of techniques. The underlying assumption of this path is that the synthesis of the best of two or more theoretical approaches offers richer possibilities than restricting practice to a single theory. Integrative counseling is an intentional process of selecting concepts and methods from a variety of therapeutic systems. An integrative perspective is well suited to meeting the diverse needs of members that are typically found in many groups.

Knowing when and how to use a particular therapeutic intervention in a group session is an art as well as the result of applying knowledge to practice. Begin by asking yourself the following questions:

- What is occurring in the group from moment to moment?
- What kind of relationships am I developing with the members of my group?
- What role(s) do I take in the group, and how does this influence group process?

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- What techniques am I drawn to, and can I explain why I am using various techniques?
- How do I evaluate my client's strengths and resources, define problems and solutions, and think about desired outcomes?

The answers to these questions will help you begin to formulate a picture of who you are as a group leader.

I (Cindy) draw upon a variety of theories, but I maintain a thread of continuity in who I am, how I make sense of my client's world, and how I intervene with a client's story. I am most influenced by the theories of Gestalt therapy and multicultural counseling and therapy, but I also utilize cognitive behavioral interventions and systemic thinking. I am very focused on the relationship I have with my clients. I believe that their style of relating to me represents what they do in the outside world. I also use myself and my reactions toward my clients as feedback, which is rooted in a Gestalt orientation. For example, I might ask clients to observe specific thoughts or behaviors they want to change. This cognitive behavior technique is grounded in my Gestalt approach, which means that the intervention would not be a predetermined assignment but would come from the spontaneous, here-and-now interactions with my clients.

In addition, I often find myself viewing clients' dilemmas through a multicultural and systemic lens, in which I see them and their presenting problems as embedded within other systems. Drawing from the feminist therapy approach, I place value on attending to how gender and power dynamics operate in a group, and I tend to avoid diagnosis without consideration of systemic, cultural, and dynamic influences. This helps me to stay away from victim blaming, and clients can see that their symptoms are often contextual. I find this helps to empower the individual to make life changes in spite of the challenges and resistance from outside forces.

A Thinking, Feeling, and Behaving Model We (Marianne and Jerry) base our practice on an integrative approach. When leading a group, we pay attention to what group members are thinking, feeling, and doing. This entails attending to the cognitive, affective, and behavioral domains. There is a reciprocal interaction between what people are thinking, and how these thoughts influence what they are feeling and doing. What we are feeling can also affect our thinking and behavior. How we behave has an influence on what we are feeling and thinking. Combining these three domains is the basis for a powerful and comprehensive approach to counseling practice. From our perspective, if any of these dimensions is excluded, the therapeutic approach is incomplete.

Both of us (Marianne and Jerry) draw from approaches in the *cognitive domain*, which focuses on the *thinking* or *thought processes* of group members. We typically challenge members to think about early decisions they have made about themselves. We pay attention to members' self-talk: "How are members' problems actually caused by the assumptions they make about themselves, about others, and about life?" "How do members create their problems by the beliefs they hold?" "How can they begin to be more self-directed by critically evaluating the sentences they repeat to themselves?" Many of our group techniques are designed to tap

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members' thinking processes, to help them reflect on events in their lives and how they have interpreted these events, and to explore their beliefs so that they can change in the direction they wish. We tend to draw much more from the cognitive behavioral approaches during the initial stage, when we are helping members to identify specific goals that will guide their participation, and then again during the ending stage, when we assist members in consolidating their learning and designing action plans to implement in everyday life.

The *affective domain* focuses on the *feelings* of group members. In the groups we lead, we help members identify and express their feelings. If members are able to experience the range of their feelings and talk about how certain events have affected them, their healing process is facilitated. If members feel listened to, understood, and accepted, they are more likely to express their feelings. Although group members can benefit from expressing repressed feelings, some cognitive work is essential if the maximum benefit is to be gained. Thus we integrate cognitive and affective work in our groups. We invite members to reflect upon their emotional work and to discuss how their emotional experience is connected to some of their beliefs about themselves, others, and life events.

The cognitive and affective domains are essential parts of the therapeutic process, but the *behavioral domain* (*acting* and *doing*) is central to the change process. Gaining insights and expressing pent-up feelings are oftentimes important, yet at some point members need to get involved in an action-oriented program of change. Group leaders can ask members useful questions such as these: "What are you doing?" "Does your present behavior have a reasonable chance of getting you what you want now, and will it take you in the direction you want to go?" If the focus of group work is on what people are doing, chances are greater that members will be able to change their thinking and feeling.

In addition to highlighting the thinking, feeling, and behaving dimensions, we help group members in consolidating what they are learning and encourage them to apply these new behaviors to situations they encounter every day. Some strategies we use are contracts, homework assignments, action programs, self-monitoring techniques, support systems, and self-directed programs of change. These approaches all stress the role of commitment on the members' part to practice new behaviors, to follow through with a realistic plan for change, and to develop practical methods of carrying out this plan in everyday life.

People in a group cannot be understood without considering the various systems that affect them—family, social groups, community, church, and other cultural forces. For a group process to be effective, it is critical to understand how individuals influence and are influenced by their social world. Effective group leaders need to acquire a holistic approach that encompasses all facets of human experience. By observing how members interact in the group, group leaders gain insight into how members may interact with other systems in their lives.

Underlying our integrative emphasis on thinking, feeling, and behaving is our (Marianne and Jerry's) philosophical leaning toward the existential approach, which places primary emphasis on the role of choice and responsibility in the therapeutic process. We invite people to look at the choices they *do* have, however limited they may be, and to accept responsibility for choosing for themselves. Thus

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we encourage members to identify and clarify what *they* are thinking, feeling, and doing as opposed to trying to change other people in their lives. Most of what we do in our groups is based on the assumption that people have the capacity to take a significant role in changing situations in their lives. We help members discover their inner resources and learn how to use them in resolving their difficulties. We do not provide answers for those in our groups, but we facilitate a process that will lead them to greater awareness of the knowledge and skills they can draw on to solve both their present and future problems.

An effective therapeutic relationship fosters a creative spirit that focuses on developing techniques aimed at increasing awareness, which enables group members to change their thinking, feeling, and behaving. Empirical evidence indicates that both the client–therapist relationship *and* the methods a therapist uses are directly related to treatment outcome. Researchers have repeatedly discovered that a positive alliance and a collaborative therapeutic relationship are the best predictors of therapy outcome (Elkins, 2016; Hubble, Duncan, Miller, & Wampold, 2010). Some neuroscience research suggests that a mutually positive connection based on empathic understanding helps set the stage for the neural integration of the brain, which promotes change in the way clients are able to deal with problematic issues (Badenoch, 2008; D. Siegel, 2010).

Part of the therapeutic relationship involves listening to client feedback about the therapy process. Client feedback can be used to inform, guide, and evaluate the treatment process. The active role members assume in a group, along with their experience of meaningful change in the early stage of a group, are the most potent contributors to positive outcome. One of the best ways to improve the effectiveness of any form of psychotherapy is by taking direction from clients through client-directed, outcome-informed therapy (Duncan, Miller, & Sparks, 2004; Miller et al., 2010).

As we introduce techniques in a group, we take into account an array of factors about the client population. We consider the members' readiness to confront an issue, the members' cultural background and value system, and the members' trust in us as group facilitators. A general goal that guides our practice is helping members identify and experience whatever they are feeling, identifying ways in which their assumptions influence how they feel and behave, and experimenting with alternative modes of behaving. We have a rationale for using the techniques we employ, and our interventions generally flow from some aspects of the theories that we describe in this chapter.

This discussion of our theoretical orientation provides only a brief introduction to this topic. For a more detailed discussion of the various theoretical orientations as they are applied to group counseling, see *Theory and Practice of Group Counseling* (Corey, 2016). For other works on theories of counseling we suggest *Case Approach to Counseling and Psychotherapy* (Corey, 2013b), *Theory and Practice of Counseling and Psychotherapy* (Corey, 2017), *Counseling and Psychotherapy: Theories and Interventions* (Capuzzi & Stauffer, 2016), *Current Psychotherapies* (Wedding & Corsini, 2014), *Contemporary Psychotherapies for a Diverse World* (Frew & Spiegler, 2013), *Counseling Theory and Practice* (Neukrug, 2011), *Systems of Psychotherapy: A Transtheoretical Analysis* (Prochaska & Norcross (2014), and *Theories of Psychotherapy and Counseling: Concepts and Cases* (Sharf, 2016).

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Developing Your Own Theory of Group Practice

Your theory needs to be appropriate for your client population, setting, and the type of group you offer. But a theory is not something divorced from you as a person. Ultimately, the most meaningful perspective is one that is an extension of your personality. At best, a theory becomes an integral part of the person you are and an expression of your uniqueness.

Throughout this book we stress that your ability to draw on your life experiences and personal characteristics is a powerful therapeutic tool. Particularly important is your willingness to become aware of and examine how your behavior, personality, cultural background, status, and position of privilege might either enhance or hinder your work as a group leader. A thorough understanding of theories applicable to group work, skill acquisition, and supervised experience provide a basic foundation for becoming an effective group leader. However, you also must be willing to take an honest look at your own life to determine if you are willing to do for yourself what you expect of your group members. If you are currently a student-in-training, developing an integrated, well-defined theoretical model will likely require extensive reading and years of practice leading groups. We suggest you exchange ideas with other group workers and modify old practices to fit new knowledge, making changes over time.

In my teaching, I (Jerry) encourage students to learn as much as they can about the various theories, and then begin to look for ways of drawing from several approaches to develop their own integrative perspective that guides their practice. For a detailed treatment of developing an integrative approach, see *The Art of Integrative Counseling* (Corey, 2013a).

Learning in Action

Identifying Your Personal Theory

To develop your personal theory of counseling, it is essential to examine your worldview, beliefs, and personal history pertaining to how people grow, heal, and change. Many students struggle to find the theories that are most congruent with their beliefs and behaviors. We have a strong sense of our own personal theory of counseling, and this has been useful in our work with beginning helpers. The following examples illustrate some ways in which life experiences leave us with beliefs about the world. You will find that the different counseling theories presented in this chapter are more likely to resonate with particular life experiences and the resulting worldview of that person. Think about what your own life experiences have taught you as you examine these examples.

Example 1

Life Experience: While an undergraduate, Finn recalls being involved in several emotionally and physically abusive relationships. She described feeling traumatized and has found it difficult to heal from these relationships.

Life Lesson: "I was stuck and wanted an apology from the person who hurt me, but I was not going to get what I needed from that person. At some point I realized that I didn't have to heal with the same person who had hurt me. This freed me to move on in my life. Instead of waiting for the person who hurt me to acknowledge wrongdoing or to apologize, I took my power back and healed myself through other relationships."

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Application to Counseling Theory: A number of counseling theories focus on the healing that occurs in relationship. People who are wounded in relationship can heal in relationship. As group leaders, we can create a healthy relationship with our clients or between group members to help them heal old wounds. Person-centered therapists place considerable emphasis on the therapeutic relationship based on the assumption that clients can apply what they are experiencing in their therapy to their personal lives. Take a moment to look over the counseling theories in this book and identify those that emphasize the power of relationships in the healing process.

Example 2

Life Experience: Ryan, a 35-year-old professional, was living under chronic stress due to an unhealthy relationship, unresolved resentment and anger, and a demanding yet unrewarding job. He struggled with many physical symptoms, including headaches, chronic respiratory infections, stomach ulcers, digestive problems, sleep problems, weight gain, depression, anxiety, and eventually cancer.

Life Lesson: "My body was sending me warning signals that I did not pay enough attention to. My physical illnesses culminated in cancer, which finally got my attention. It was time to change the way I was living. The lesson I took from this time in my life is, 'If you don't scream, your body will'."

Application to Counseling Theory: Several theoretical orientations focus on a holistic approach to working with clients, paying attention to clients' physical symptoms, mannerisms, and behaviors. A Gestalt therapist might see neck pain in a client as a symptom of something or someone literally being a pain in the neck in that person's life. The Gestalt therapist helps clients listen to their body and engages them in a dialogue with their physical symptoms. Cognitive behavior therapy also works holistically by attending to the integration of thinking, feeling, and behaving dimensions. A practitioner with a cognitive behavior therapy orientation might help clients identify how their beliefs and thinking patterns have influenced what they are doing and how they are feeling.

Directions

Take a few minutes to reflect on significant events or critical incidents that have occurred in your life. It could be your parents' divorce, an abusive relationship, losing a job, having a child, getting sober, or some other event.

Life Experience: Briefly describe the event or incident, when it occurred, what happened, and why it was significant or life-changing for you.

Life Lesson: What was the outcome of this event, and how did it affect you emotionally, cognitively, and behaviorally? How did it influence the way you see the world or others? What belief or worldview did you develop from this experience?

Theoretical Application: With the help of your instructor or with classmates, discuss which counseling theories are most compatible with the belief or worldview you now hold as a result of this life experience. Try this exercise with several life experiences to see if the resulting beliefs and life lessons lead you in the direction of specific counseling theories.

LO2

Using Group Techniques Effectively

Techniques are leader interventions aimed at facilitating movement within a group. Virtually anything a group leader does could be viewed as a technique, including being silent, suggesting a new behavior, inviting members to deal with a

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conflict, offering feedback to members, presenting interpretations, and suggesting homework assignments to be done between group sessions. A **technique** refers to a leader's explicit and directive request of a member (or members) for the purpose of focusing on material, augmenting or exaggerating affect, practicing behavior, or consolidating insight and new learning. Techniques may include asking a group to clarify the direction it wants to take when the group seems to lose energy, inviting members to share a significant story, asking a member to role-play a specific situation, asking a member to practice a new behavior, asking members to bring past events into the here and now, encouraging a person to repeat certain words or to complete a sentence, helping members summarize what they have learned from a group session, working collaboratively with members in designing homework assignments, challenging a member's belief system, and working with the cognitions that influence a member's behavior.

In facilitating a group, we use a variety of techniques drawn from many theoretical models. Techniques are adapted to the needs of the group participants, and we consider several factors: the purpose and type of group, readiness of members to deal with a personal concern, cultural background, value system, and trust in us as leaders. We also consider the level of cohesion and trust among group members when deciding on appropriate interventions. We encourage group participants to become aware of whatever they are feeling, identify the ways their assumptions influence how they feel and behave, and experiment with alternative modes of behaving.

We also consider as techniques those procedures aimed at helping group leaders get a sense of the direction they might pursue with a group. In many types of groups, the most useful techniques grow out of the work of the participants and are tailored to situations that evolve in a particular session. Techniques are the *tools* and interventions used to facilitate what is going on in a group. For example, if the group members are being silent, the leaders may ask each person to complete the sentence, "Right now one thing I am silent about is . . .".

Rationale for Use of Techniques

Techniques are most useful when they evolve from the work of the group participants and are tailored to the situations that evolve in a particular group meeting. As group leaders, it is imperative that we consider the rationale underlying the use of a particular technique and consider whether the technique is likely to foster the client's self-exploration and self-understanding. If a supervisor or coleader were to ask us about our rationale for using a technique, we should be able to answer these questions:

- Why did we use this particular technique?
- What did we hope to accomplish by using a technique?
- What did we expect the group members would learn from the intervention?
- What theoretical framework guided our choice of a technique?

Consider the following case as a basis for reflecting on the purpose of having a theoretical justification for making interventions in a group.

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Rebecca, a trainee leading a small group, was asked by the supervisor, "What was your rationale for introducing the exercise during a group session when the energy seemed low and members were reluctant to participate?" Rebecca was somewhat defensive and said she was following her intuition that what the group needed was a booster shot to get moving again. When the leader explored with Rebecca how her theory might influence the interventions she makes, she replied: "I don't think a theory is really necessary. I have a tool kit of techniques, and I feel confident in drawing from them when they are needed. My preference is to go with the flow of the group rather than imposing my theory on a group."

The supervisor explained to Rebecca that a theory provides a general framework that can help her make sense of what is going on in the group, and that a theory also can provide a sense of direction for what she says and does in a group. Rebecca insists that trying to fit into a theory limits her creativity and is likely to result in mechanical interventions. Rebecca sees little value in having a theory to guide her work. She would rather rely on her clinical hunches and create interventions that "feel right."

What are your reactions to what Rebecca is saying? It is possible to trust your clinical intuitions and at the same time to reflect on the conceptual basis for your interventions. Theory can inform and provide a structure for expressing your clinical impressions and interpretations of what is going on in a group. If a supervisor asked you what your preferred theory was, would you be able to describe it? To what extent do you think the techniques you employ in a group need to have some theoretical basis?

It is important to use techniques you have some knowledge about, preferably those you have experienced personally or have received supervision in using. We use the following guidelines in our practice to increase the effectiveness of techniques or any structured exercises that we might introduce in a group session:

- Techniques used have a therapeutic purpose and are grounded in some theoretical framework.
- Techniques (and exercises) are presented in an invitational manner; members are given the freedom either to participate in or to not participate in a given experiment.
- Techniques are introduced in a timely and sensitive manner and are abandoned if they are not working.
- Techniques are modified so that they are suitable for the client's cultural and ethnic background.
- Participants have an opportunity to share their reactions to the techniques or activities used.
- The client's self-exploration and self-understanding is fostered.

LO3

Viewing a Group Through a Multicultural Lens

All people are comprised of multiple identities, and layers of cultural influences are evident in their behaviors. To work effectively with culturally diverse group members, all dimensions of identity including age, gender, sexual orientation,

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social status, ability, and religious affiliation must be considered regardless of your theoretical perspective. Leaders need to be aware of the power frequently attributed to them by group members and recognize that they may symbolize various institutions of oppression for some members. When introducing exercises or suggesting experiments, it is essential for leaders to be mindful of being invitational and not abusing the authority and power often given to them by group members.

If experiences with discrimination and oppression are not validated and understood, it is highly unlikely that a leader will be able to develop trust and build rapport with people of color or members of other oppressed groups. The pain that the members encounter must be addressed before challenging them to respond differently. Some theoretical approaches challenge clients to examine the way they are interpreting events in their lives and to assume control over their thoughts and feelings. Other theories place a great deal of emphasis on identifying and expressing emotion.

Members of some cultural groups might find the goal of emotional expression to be incompatible with their cultural upbringing and gender-role norms. For example, if a male group member has been taught not to express his feelings in public, it may be both inappropriate and ineffective to quickly introduce techniques aimed at bringing feelings to the surface. In this case it is important to first find out whether this member is interested in exploring what he has learned from his culture about expressing his feelings. Leaders must respect the cultural values and experiences of members, yet at the same time they may invite members to think about how their values, life experiences, socialization, and upbringing continue to affect their behavior and the choices they are making. If members indicate they want to talk about how their cultural background is affecting them today, then leaders can assist members in examining the pros and cons of making behavioral changes and the cost of doing so.

Western therapeutic models reflect values such as choice, the uniqueness of the individual, assertiveness, personal growth, and a strong ego. Therapeutic outcomes include improving assertive coping skills by changing the environment, changing one's coping behavior, and learning to manage stress. Western therapeutic models—such as the cognitive behavioral therapies and the relationship-oriented approaches—are oriented toward individual change. In contrast, many non-Western cultures focus more on the social framework than on development of the individual and teach the value of interdependence, play down individuality, think of the collective good, and emphasize healing in community. Asians, Latinos/as, and African Americans often come from collectivistic cultures. Many Asian cultures stress self-reliance, and some Asian group members may remain quiet rather than disclose their personal problems or reveal personal details about their family (Chung & Bemak, 2014). The goals, structure, and techniques used in a group need to be modified to make a group culturally appropriate and inclusive of members' various worldviews. In an increasingly pluralistic society, there is an ethical imperative to avoid forcing all clients to fit a mold that may not be appropriate for their cultural background. Even within Western cultures norms may differ. For example, Native American cultures emphasize family and community and place less emphasis on individualism.

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As group counselors, we need to be aware of how our assumptions and underlying theoretical orientation influence practice with diverse clients. Just as we as group leaders need to attend to our cultural effectiveness with clients, so too do the theories and techniques we use need to be adjusted and tailored to suit the members' cultural and ethnic backgrounds. Being willing to tailor our methods to the needs of members increases the chances of creating positive outcomes.

LO4

Relationship of Theories to Techniques

Some techniques cut across a variety of theories, and others are linked to particular theoretical approaches. The following sections consider the key concepts and techniques of a number of counseling theories, which we have grouped into four general categories:

1. **Psychodynamic approaches** stress insight in therapy (psychoanalytic and Adlerian therapy).
2. **Experiential and relationship-oriented approaches** stress feelings and subjective experiencing (existential, person-centered, Gestalt therapy, and psychodrama).
3. **Cognitive behavioral approaches** stress the role of thinking and doing and tend to be action-oriented (behavior therapy, cognitive therapy, rational emotive behavior therapy, and reality therapy).
4. **Postmodern approaches** stress understanding the subjective world of the client and tap existing resources for change within the individual (solution-focused brief therapy, narrative therapy, motivational interviewing, and feminist therapy).

Although we have separated the theories into four general clusters, this categorization is somewhat arbitrary. Overlapping concepts and themes make it difficult to neatly compartmentalize these theoretical orientations.

In the *Groups in Action: Evolution and Challenges DVD and Workbook*, see Part Three, which consists of a lecture overview of the main theoretical approaches to group work and provides many examples of techniques that can be applied to a wide range of groups. This 1-hour lecture program covers the basic concepts of the four general theories briefly described in this chapter.

LO5

Psychodynamic Approaches

First we consider the psychodynamic approaches to group counseling. **Psychoanalytic therapy** is based largely on insight, unconscious motivation, and reconstruction of the personality. The psychoanalytic model has had a major influence on all of the other formal systems of psychotherapy. Some of the therapeutic models we consider are basically extensions of the psychoanalytic approach, others are modifications of analytic concepts and procedures, and still other theories emerged as a reaction against psychoanalysis. Many theories of counseling and psychotherapy

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have borrowed and integrated principles and techniques from psychoanalytic approaches.

Adlerian therapy differs from psychoanalytic theory in many respects, but it can broadly be considered from a psychodynamic perspective. Adlerians focus on meaning, goals, purposeful behavior, conscious action, belonging, and social interest. Although Adlerian theory accounts for present behavior by studying childhood experiences, it does not focus on unconscious dynamics.

Psychoanalytic Approach

Key Concepts of Psychoanalytic Therapy The psychoanalytic approach views people as being significantly influenced by unconscious motivation and early childhood experiences. Because the dynamics of behavior are buried in the unconscious, group therapy often consists of a lengthy process of analyzing inner conflicts that are rooted in the past. Longer-term analytic group therapy is largely a process of restructuring the personality, which has broader treatment goals than most approaches. Brief psychodynamic group therapy approaches address more modest goals in a limited time frame.

Psychoanalytic group therapy focuses on the influence of the past on current personality functioning. Experiences during the first 6 years of life are seen as the roots of one's problems in the present. Contemporary analytically oriented group practitioners are interested in their members' past, but they intertwine that understanding with the present and with the future. The past is relevant only as it influences the present and the future, and in this sense all three have an essential place in group therapy (Rutan, Stone, & Shay, 2014).

Therapeutic Goals of Psychoanalytic Therapy A primary goal is to make the unconscious conscious. Rather than solving immediate problems, the goal is the restructuring of personality. Successful outcomes of psychoanalytic therapy result in significant modification of an individual's personality and character structure.

Therapeutic Relationship Leadership styles vary among psychoanalytically oriented group therapists, ranging from leaders characterized by objectivity, warm detachment, and relative anonymity to those who favor a role that is likely to result in a collaborative relationship with group members. A significant development of psychoanalytically oriented group therapy is the growing recognition of the central importance of the therapeutic relationship. In contrast to the classical model of the impersonal and detached analyst, the contemporary formulation emphasizes the therapeutic alliance. Establishing a working relationship in which the therapist communicates caring, interest, and involvement with members is now the preferred model.

Techniques Major techniques include maintaining the analytic framework, free association, interpretation, dream analysis, analysis of resistance, and analysis of transference. These techniques are geared to increasing awareness, acquiring insight, and beginning a working-through process that will lead to a reorganization of the personality. Two key features of psychodynamic group therapy are the

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ways transference and countertransference play out in the context of the current group situation.

Groups offer many opportunities for the exploration of transference reactions that have roots in prior relationships. The group constellation lends itself to **multiple transferences** that provide for reenacting past unfinished events, especially when other members stimulate such intense feelings in an individual that he or she "sees" in them some significant figure such as a father, mother, sibling, life partner, spouse, ex-lover, or boss. A basic tenet of psychodynamic therapy groups is the idea that group members, through their interactions within the group, re-create their social situation, implying that the group becomes a microcosm of their everyday lives. The leader has the task of helping members discover the degree to which they respond to others in the group as if they were their parents or siblings. A psychoanalytic group provides a safe, neutral environment in which members can express spontaneous thoughts and feelings, and the group is a conducive milieu in which to relive significant past events. For example, if a female group member has an intense reaction to a male member, she may discover that she is projecting some old hurts from a critical father onto this member. If both members are willing to explore her transference, she can gain insight into ways that she brings her past relationship with her father into present relationships with certain males.

Groups can provide a dynamic understanding of how people function in out-of-group situations. By reliving the past through the transference process, members gain increased awareness of the ways in which the past is obstructing present functioning. By interpreting and working through their transferences, participants become increasingly aware of the ways in which past events interfere with their ability to appraise and deal with reality in everyday life.

The other side of members' transferences is the countertransference of the group leader, whose feelings may become entangled in the therapeutic relationship with members, obstructing objectivity. Countertransference can be viewed as the group therapist experiencing feelings from the past that are reactivated by the group member in the present (Rutan et al., 2014). In a broader sense, countertransference involves the group therapist's total emotional response to a member. To the degree that countertransference is present, group therapists react to members as if they were significant figures of their own original family. Group leaders need to be alert to signs of their unresolved conflicts that could interfere with their leadership and create a situation in which members are used to satisfy their own unfulfilled needs. For example, a group leader who feels unappreciated by others in his personal life may experience difficulty working with members who are demanding and communicate that he is failing to meet their needs. The difficulty in recognizing one's own countertransference and the necessity that such reactions be acknowledged and therapeutically dealt with provide a rationale for group leaders to experience their own therapy. Personal therapy is valuable in helping leaders to recognize signs of countertransference and in discovering how their own needs and motivations influence their group work. It is critical that countertransference be managed and used for the benefit of working with the group members. When group therapists study their own internal reactions and use them to understand the members of their groups, countertransference can greatly benefit the therapeutic work.

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Multicultural Applications of the Psychoanalytic Approach

With this orientation emphasis is placed on how group members' past experiences have a contemporary influence on their personality. The briefer forms of psychodynamic therapy have particular relevance in taking into consideration clients' cultural context and ways that early experiences can provide a new understanding of current problems. With this briefer form of psychoanalytically oriented therapy, clients can relinquish old patterns and establish new patterns in their present behavior.

To learn more about the psychodynamic approach to group practice, we recommend *Psychodynamic Group Psychotherapy* (Rutan et al., 2014).

The Adlerian Approach

Key Concepts of Adlerian Therapy According to the Adlerian approach, people are primarily social beings, influenced and motivated by societal forces. Human nature is viewed as creative, active, and decisional. The approach focuses on the unity of the person, on understanding the individual's subjective perspective, and on the importance of life goals that give direction to behavior. Adler holds that inherent feelings of inferiority initiate a natural striving toward achieving a higher level of mastery and competence in life. The subjective decisions each person makes regarding the specific direction of this striving form the basis of the individual's lifestyle (or personality style). The style of life consists of our views about others, the world, and ourselves; these views lead to distinctive behaviors that we adopt in pursuit of our life goals. We can influence our own future by actively and courageously taking risks and making decisions in the face of unknown consequences. Individuals are not viewed as being "sick" or suffering from a psychopathological disorder and needing to be "cured."

Both psychoanalytic and Adlerian groups often replicate the original family in many respects, which enables members to reexperience conflicts that originated in their family. Because of the family-like atmosphere, the group provides opportunities to evoke associations to both family-of-origin and present life experiences (Rutan et al., 2014). Adlerians place emphasis on the family constellation as a key factor in influencing one's style of life. The group is viewed as a representation of society, which allows members to better understand their own dynamics by seeing patterns of family dynamics emerge within the group.

In Adlerian groups the members are able to bring their personal history into the present group. Members can gain practice acquiring new behavioral patterns by acting *as if* they had made some of the changes they most desire. For example, Darlene struggles with feeling inferior and is unable to express herself effectively both in her daily life and in the group. Interestingly, most members view Darlene as extremely bright and able to communicate clearly—when she does participate. They encourage her to speak more often because they like what she has to say. The group leader suggests to Darlene that she experiment in one group session by *acting as if* she has all the attributes other members see in her. As a result of doing this experiment, Darlene begins to recognize and appreciate her talents. She is encouraged to continue practicing this new behavior for one week in her outside life and to report to the group what the experience was like for her.

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Therapeutic Goals of Adlerian Counseling A key goal of an Adlerian group is fostering **social interest**, or facilitating a sense of connectedness with others. Adlerian leaders want to cultivate in members the desire to contribute to the welfare of others, to enhance a sense of identification and empathy with others, and to enhance a sense of belongingness with a group. To accomplish this goal, an Adlerian leader creates a democratic climate within the group. Adlerians do not screen members for their groups because this is viewed as being inconsistent with the spirit of democracy and equality. Sonstegard and Bitter (2004) contend that the screening and selection process itself fails to provide an opportunity for those who most need a group experience. They believe screening is more often done for the comfort of the group leader than for ensuring that those who can most benefit from a group are included.

Counseling is not simply a matter of an expert therapist making prescriptions for change. It is a collaborative effort, with the group members and the group leader working together on mutually accepted goals. The members recognize that they are responsible for their behavior. Adlerians are mainly concerned with challenging clients' mistaken notions and faulty assumptions. Therapy provides encouragement and assists group members in changing their cognitive perspectives and behavior.

Therapeutic Relationship Adlerians base their therapeutic relationship on cooperation, mutual trust, respect, confidence, collaboration, and alignment of goals. They especially value the group leader's modeling of communication and acting in good faith. From the beginning of a group, the relationship between the leader and the members is collaborative and is characterized by working toward specific, agreed-upon goals. Adlerian group therapists strive to establish and maintain an egalitarian therapeutic alliance and a person-to-person relationship with the members of their groups.

Techniques Adlerians have developed a variety of techniques and therapeutic styles. Adlerians are not bound to follow a specific set of procedures; rather, they can tap their creativity by applying those techniques that they think are most appropriate for each client. Some of the specific techniques they often employ are attending, providing both confrontation and support, summarizing, gathering life-history data, lifestyle analysis, interpretation of experiences within the family and early recollections, suggestion, offering encouragement, homework assignments, and assisting group members in searching for new possibilities. Adlerian therapy has a psychoeducational focus, a present and future orientation, and is a brief or time-limited approach.

Interpretation is a key technique of Adlerian group counselors and involves the leader addressing members' underlying motives for behaving the way they do in the here and now. Interpretations are never forced on group members but are presented tentatively in the form of hypotheses, as shown in these examples: "Could it be that your frequent reaction of feeling ignored in this group has something to do with being unnoticed by your family when you were a child?" "I have a hunch that I'd like to share with you." "It seems to me that you are more willing to help others in this group than you are to attend to yourself." "Perhaps you

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could entertain the idea that waiting to the end of a session to bring up your concerns results in you sabotaging yourself." "I get the impression that no matter how many times people tell you that they like to hear from you, you continue to convince yourself that you have nothing to say."

Interpretations are open-ended presentations of clinical hunches that can be explored in group sessions. Interpretations are best achieved collaboratively within groups with group members offering hunches about possible meanings of their own behavior. The ultimate goal of this process is that participants will come to a deeper psychological understanding of themselves. The aim is for members to acquire deeper awareness of their own role in creating a problem, the ways in which they are maintaining the problem, and what they can do to improve their life situation.

During the advanced stage of an Adlerian group (the reorientation stage), the members are encouraged to take action based on what they have learned from their group participation. The group becomes an agent of change because of the improved interpersonal relations among members. The group process enables members to see themselves as others do and to recognize faulty self-concepts or mistaken goals that they are pursuing. Change is facilitated by the emergence of hope.

During the action stage of an Adlerian group, members make new decisions and their goals are modified. To challenge self-limiting assumptions, members are encouraged to **act as if** they were already the person they would like to be. Adlerians often use this action-oriented technique as a way to facilitate shifting one's view of a situation, enabling members to reflect on how they could be different.

Group members may be asked to "catch themselves" in the process of repeating old patterns that have led to ineffective or self-defeating behavior, such as having many reactions in a session, yet not expressing them verbally. The technique of **catching oneself** involves helping individuals identify signals associated with their problematic behavior or emotions. If participants hope to change, they need to set tasks for themselves and do something specific about their problems. Furthermore, commitment is needed to translate new insights into concrete action.

Adlerian brief group counseling applies a range of techniques within a time-limited framework. Characteristics of this brief approach include initial establishment of a therapeutic alliance, identifying target problems and goal alignment, rapid assessment, active and directive interventions, a focus on strengths and resources of the members, and an emphasis on both the present and the future. Adlerians are flexible in adapting their interventions to each group member's unique life situation.

Multicultural Applications of the Adlerian Approach This approach offers a range of cognitive and action-oriented techniques to help people explore their concerns in a cultural context. Adlerians' interest in helping others, in social interest, in pursuing meaning in life, in belonging, and in the collective spirit fits well with the group process. This approach respects the role of the family as influential in personality development and stresses social connectedness and establishing meaningful relationships in a community. Adlerian therapists tend to focus on cooperation and socially oriented values as opposed to competitive and individualistic

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values. Adlerian practitioners are flexible in adapting their interventions to each client's unique life situation. The approach has a psychoeducational focus, a present and future orientation, and is a brief, time-limited approach. All of these characteristics make the Adlerian approach to group counseling suitable for working with a wide range of client problems.

An excellent resource for learning more about the Adlerian approach to group work is *Adlerian Group Counseling and Therapy: Step-by-Step* (Sonstegard & Bitter, 2004).

LO6

Experiential and Relationship-Oriented Approaches

Therapy is often viewed as a journey taken by counselor and client, a journey that delves deeply into the world as perceived and experienced by the client. This journey is influenced by the quality of the person-to-person encounter in the therapeutic situation. The value of the therapeutic relationship is a common denominator among all therapeutic orientations, yet some approaches place more emphasis than others do on the role of the relationship as a healing factor. Both existential therapy and person-centered therapy place central prominence on the person-to-person relationship. Emphasizing the human quality of the therapeutic relationship lessens the chances of making group counseling a mechanical process. It is not the techniques we use that make a therapeutic difference; rather, it is the quality of the relationships with group members that heals. If the members of our groups are able to sense our presence and our intention to make a real connection, then a solid foundation is being created for the hard work that follows.

The relationship-oriented approaches (sometimes known as experiential approaches) are all based on the premise that the quality of the therapeutic relationship is primary, with techniques being secondary. Under this general umbrella are *existential therapy, person-centered therapy, Gestalt therapy,* and *psychodrama.* With the experiential and relationship-oriented approaches, group leaders are not bound by a specific set of techniques. They use techniques in the service of broadening the ways in which group members live in their world. Techniques are tools to help the members become aware of their choices and their potential for action.

The experiential approaches are grounded on the premise that the therapeutic relationship fosters a creative spirit of inventing techniques aimed at increasing awareness, which allows individuals to change some of their patterns of thinking, feeling, and behaving.

Some of the key concepts common to all experiential approaches include the following:

- The quality of the leader-to-member encounter in the therapeutic situation is the catalyst for positive change.
- Emphasis is placed on the leader's ability to establish a climate that fosters authentic interchanges among the members.

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- The I/Thou relationship (a genuine person-to-person connection) enables members to experience the safety necessary for risk-taking behavior.
- Awareness emerges within the context of a genuine meeting between the leader and the members, or within the context of authentic relating.
- The group leader's main role is to be present with members during the group time. One way to increase this presence is through appropriate self-disclosure by the leader.
- Members can best be invited to behave authentically by a group leader modeling authentic behavior.
- A therapist's attitudes and values are at least as critical as are his or her knowledge, theory, or techniques.
- A group leader who is not sensitively tuned in to his or her own reactions to the members may become more a technician than a skilled facilitator.
- The basic work in a group is done by the members. A leader's job is to create a climate in which members are likely to try out new ways of being.
- Attending to feelings is a useful route to changing one's thinking and behaving.

Counselors who operate in the framework of the relationship-oriented therapies are much less anxious about using the "right technique." Their techniques are most likely designed to enhance some aspect of the client's experiencing, rather than being used to stimulate clients to think, feel, or act in a certain manner.

The Existential Approach

Key Concepts of Existential Therapy The **existential perspective** holds that we define ourselves by our choices. Although outside factors restrict the range of our choices, we are ultimately the authors of our lives. Because we have the capacity for awareness, we are basically free. Along with our freedom, however, comes responsibility for the choices we make. Our task is to create a meaningful existence. Existential group practitioners contend that clients often lead a "restricted existence," seeing few if any alternatives for dealing with life situations and tending to feel trapped or helpless. A group experience can help members recognize outmoded patterns of living and accept responsibility for changing their future. For instance, if you have suffered great rejection in your childhood, you may cling to a view of yourself as being unacceptable, in spite of the fact that many group members are accepting of you and claim they admire you.

There are six key propositions of existential therapy: (1) We have the capacity for self-awareness. (2) Because we are basically free beings, we must accept the responsibility that accompanies our freedom. (3) We have a concern to preserve our uniqueness and identity; we come to know ourselves in relation to knowing and interacting with others. (4) The significance of our existence and the meaning of our life are never fixed once and for all; instead, we re-create ourselves through our projects. (5) Anxiety is part of the human condition. (6) Death is also a basic human condition, and the reality of our mortality heightens our sense of ultimate aloneness. The reality of death can lead to increased awareness that we do not have forever to actualize our being. Death awareness can give significance to living.

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Therapeutic Goals of Existential Therapy The principal goal of an existential group is to assist the participants in recognizing and accepting the freedom they have to become the authors of their own lives. Group leaders encourage members to examine the ways in which they are avoiding their freedom and the responsibility that accompanies it.

The existential group represents a microcosm of the world in which participants live and function. It is assumed that over time the interpersonal and existential problems of the participants become manifest in the here-and-now interactions within the group. The central purpose of this kind of group is to enable members to discover themselves as they are by sharing their existential concerns.

Therapeutic Relationship The existential approach places primary emphasis on understanding members' current experience, not on using therapeutic techniques. The therapeutic relationship is of paramount importance, for the quality of the I/Thou encounter (interpersonal relationships in the group) offers a context for change. Existential group therapists value being fully present and strive to create caring relationships with the members of their groups. Therapy is a collaborative relationship in which both members and the leader are involved in a journey of self-discovery.

Techniques Existential therapy reacts against the tendency to view therapy as a system of well-defined techniques; it affirms looking at those unique characteristics that make us human and building therapy on them. Existential group therapists are free to adapt their interventions to their own personality and style, as well as paying attention to what each group member requires. Existential group leaders are *not* bound by any prescribed procedures and can use techniques from other therapy schools; however, their interventions are guided by a philosophical framework about what it means to be human.

Multicultural Applications of the Existential Approach Because the existential approach is based on universal human themes, and because it does not dictate a particular way of viewing reality, it is highly applicable when working in a multicultural context. Themes such as relationships, finding meaning, anxiety, suffering, and death are concerns that transcend the boundaries that separate cultures. Clients in existential group therapy are encouraged to examine the ways their present existence is being influenced by social and cultural factors. The existential approach is particularly relevant for multicultural situations because it does not impose particular values and meanings; rather, it investigates the values and meanings of group members. Existential group counselors respect the different elements that make up a member's philosophy of life. They respect the uniqueness of the particular situation of each client and do not impose their cultural values on the person. With this understanding of different worldviews, group practitioners are in a position to establish mutually agreed-upon goals with individuals that will provide a direction for change.

If you are interested in learning more about the existential approach to group work, we recommend *Existential-Humanistic Therapy* (Schneider & Krug, 2010), *Existential Psychotherapy* (Yalom, 1980), and *The Schopenhauer Cure: A Novel* (Yalom, 2005a).

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The Person-Centered Approach

Key Concepts of Person-Centered Therapy The **person-centered approach** rests on the assumption that we have the capacity to understand our problems and that we have the resources within us to resolve them. The group facilitator focuses on the constructive side of human nature and on what is right with people. Group members are able to change without a high degree of structure and direction from the facilitator. Group facilitators provide understanding, genuineness, support, acceptance, caring, and positive regard. This approach emphasizes fully experiencing the present moment, learning to accept oneself, and deciding on ways to change. The person-centered approach stresses the active role and responsibility of the group member. It is a positive and optimistic view and calls attention to the need to account for a person's inner and subjective experiences.

Therapeutic Goals of Person-Centered Therapy A major goal is to provide a climate of safety and trust in the therapeutic setting so that the client, by using the therapeutic relationship for self-exploration, can become aware of blocks to growth. Because this approach stresses the client–therapist relationship as a *necessary and sufficient condition* leading to change, it minimizes directive techniques, interpretation, questioning, probing, diagnosis, and collecting history. Group members are trusted to identify personally meaningful goals and to find their own way without active and directive structuring from the group leader.

Therapeutic Relationship Emphasizing the crucial role of the therapist's attitudes and personal characteristics, this approach makes the therapeutic process relationship-centered rather than technique-centered. The qualities of the facilitator that determine the relationship include the attitudes of genuineness, nonpossessive warmth, accurate empathy, unconditional acceptance of and respect for the client, caring, and the communication of those attitudes to the client.

The primary function of the facilitator is to create an accepting and healing climate in the group. Person-centered therapy is best considered as a "way of being" rather than a "way of doing." The group leader is called a *facilitator,* which reflects the importance of interactions between group members and the leader's ability to assist members in expressing themselves. Person-centered group facilitators use themselves as instruments of change in a group. Their central function is to establish a therapeutic climate in which group members will interact in increasingly authentic ways.

Person-centered therapy is best considered as a "way of being" rather than a "way of doing."

The person-centered group approach emphasizes certain attitudes and skills as a necessary part of the facilitator's style: listening in an active and sensitive way, accepting, understanding, reflecting, clarifying, summarizing, sharing personal experiences, responding, encountering and engaging others in the group, going with the flow of the group rather than trying to direct the way the group is going, and affirming a member's capacity for self-determination. Other relational qualities and attitudes embraced by person-centered therapists include receptivity to experience, contact and engagement, a therapeutic

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alliance, authentic dialogue, understanding the client's experience, and hopefulness regarding the client's capacity for the relationship (Cain, 2010).

Techniques Person-centered therapy has evolved through diversity, inventiveness, creativity, and individualization in practice. In newer versions of the person-centered approaches, group facilitators have increased freedom to participate in the relationship, to share their reactions, to confront clients in a caring way, and to be active in the therapeutic process (Cain, 2010; Kirschenbaum, 2009). Current formulations of the approach assign more importance to therapists' bringing in their own here-and-now reactions to what is occurring with a group. Doing so can motivate members to explore themselves at a deeper level. These changes from Carl Rogers's original view of the counselor have encouraged the use of a wider variety of methods and a considerable diversity of therapeutic styles. Therapists need to evolve as people rather than being intent on expanding their repertoire of techniques. However, it is important to remember that effective group facilitators need to be therapeutic people, *and* they also must have the knowledge and skills required to assist members in reaching their personal goals in a group.

Group practitioners can adapt the person-centered approach to their personal convictions and therapeutic style, integrating their own life experiences and ideas into their work with groups. Integrative personal-centered therapists subscribe to a fundamental philosophy, yet they may blend concepts and methods from existential, Gestalt, and experiential approaches in facilitating their groups.

Natalie Rogers (2011) has made a significant contribution to the application of the person-centered approach by incorporating the expressive arts as a medium to facilitate personal exploration in a group context. *Person-centered expressive arts therapy* uses various artistic forms—movement, drawing, painting, sculpting, music, writing, and improvisation—toward the end of growth, healing, and self-discovery. This is a multimodal group approach integrating mind, body, emotions, and spiritual inner resources. Individuals who have difficulty expressing themselves verbally can find new possibilities for self-expression through the various nonverbal forms of expression available to them.

Multicultural Applications of Person-Centered Therapy The emphasis on universal, core conditions provides the person-centered approach with a framework for understanding diverse worldviews. Empathy, being present, and respecting the values of group participants are particularly important attitudes and skills in groups with culturally diverse individuals. These attitudes are not limited to any one cultural group but transcend culture. Person-centered counselors convey a deep respect for all forms of diversity and value understanding the client's subjective world in an accepting and open way. One potential limitation of this approach is the fact that some clients come to a group to find solutions for pressing problems. Individuals from certain cultures may expect a directive leader who functions in an expert role as an authority, who offers advice and recommends a specific course of action, and they may experience difficulty with a leader who does not provide the structure they want.

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For more information on the person-centered approach, we recommend *The Creative Connection for Groups: Person-Centered Expressive Arts for Healing and Social Change* (N. Rogers, 2011), *Person-Centered Psychotherapies* (Cain, 2010), and *Humanistic Psychotherapies: Handbook of Research and Practice* (Cain, Keenan, & Rubin, 2016).

Gestalt Therapy

Key Concepts of Gestalt Therapy The **Gestalt approach** is an existential and phenomenological approach based on the assumption that individuals and their behavior must be understood in the context of their ongoing relationship with the present environment. The group therapist's task is to support the members as they explore their perceptions of reality. The fundamental method to assist in this exploration is awareness of the internal (intrapersonal) world and contact with the external environment. Change occurs naturally as awareness of "what is" increases. Heightened awareness can lead to a more thorough integration of parts of the group member's reality that were fragmented or unknown.

This approach focuses on the here and now, direct experiencing, awareness, bringing unfinished business from the past into the present, and dealing with unfinished business. Other concepts include energy and blocks to energy, contact, and paying attention to nonverbal cues. Group members identify their own unfinished business from the past that is interfering with their present functioning by reexperiencing past situations as though events were happening in the present moment.

Therapeutic Goals of Gestalt Therapy The primary goal of Gestalt therapy is attaining awareness and greater choice. Awareness includes knowing the environment and knowing oneself, accepting oneself, and being able to make contact. Group members are helped to pay attention to their own awareness process so that they can be responsible and can selectively and discriminatingly make choices.

Therapeutic Relationship As is true for the other experiential approaches, the focus is not on the techniques employed by the therapist but on who the therapist is as a person and the quality of the relationship. Factors that are emphasized include the therapist's presence, authentic dialogue, gentleness, direct self-expression by the therapist, and a greater trust in the client's experiencing. There are many different styles of practicing Gestalt therapy in a group, but all styles share common elements: direct experiencing and experimenting and attention to *what* and *how* and *here* and *now*.

Techniques Gestalt group leaders think more in terms of experiments than techniques. Although the therapist functions as a guide and a catalyst, presents experiments, and shares observations, the basic work of therapy is done by the group members. Group leaders do not force change on the members; rather, leaders create experiments within a here-and-now framework of what is going on in the group. These experiments are the cornerstone of experiential learning. Gestalt therapy utilizes the experiment to move group members from talk to action and experience. For example, assume a member is talking about a problematic relationship with a

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friend. The leader is likely to invite this member to bring the friend symbolically into the room, either by talking to an empty chair or talking directly to another group member as though he or she were the friend. With the emphasis given to the relationship between client and therapist, there is a creative spirit of suggesting, inventing, and carrying out experiments aimed at increasing awareness.

Although the group leader suggests the experiments, this is a collaborative process with full participation by the group members. Gestalt experiments take many forms: setting up a dialogue between a group member and a significant person in his or her life; assuming the identity of a key figure through role playing; reliving a painful event; exaggerating a gesture, posture, or some nonverbal mannerism; or carrying on a dialogue between two conflicting aspects within an individual. For effective application of Gestalt procedures, it is essential that clients be prepared for such experiments.

Sensitively staying in contact with a group member's flow of experiencing involves paying attention to the person and not on being concerned with the mechanical use of techniques to bring about a certain effect. Gestalt practitioners who have truly integrated their approach are able to apply their skills in a flexible way by adapting their methods to each individual. They have solid training in Gestalt theory and practice, which allows them to design experiments that will deepen the work of members. They strive to help members experience themselves as fully as possible in the present.

Gestalt therapy is truly an integrative orientation in that it focuses on whatever is in the individual's awareness. From the Gestalt perspective, feelings, thoughts, body sensations, and actions are all used as pathways to understand what is central for the client in each moment. The centrality of whatever is in the group member's awareness is an ideal way to understand his or her world. By paying attention to the verbal and nonverbal cues provided, a group leader has a starting point for exploring the member's world.

Multicultural Applications of Gestalt Therapy There are many opportunities to apply Gestalt experiments in creative ways with diverse client populations. Gestalt experiments can be tailored to fit the unique way in which an individual perceives and interprets his or her culture. Gestalt leaders approach each client in an open way without preconceptions. They do this by checking out their views in dialogue with the group member. This is particularly important in working with individuals from diverse cultural backgrounds. They are concerned about how and which aspects of this background become central, or figural, for them and what meaning clients place on these figures.

Because Gestalt therapy is practiced with a phenomenological attitude, therapists are less likely to impose their own values and cultural standards on their clients. Gestalt therapy can be used creatively and sensitively with culturally diverse populations if interventions are used flexibly and in a timely manner. One of the advantages of drawing on Gestalt experiments is that they can be tailored to fit the unique way in which an individual member perceives and interprets his or her culture. Before Gestalt procedures are introduced, it is crucial that the group members have been adequately prepared. Gestalt group practitioners focus on understanding the person and not on the use of techniques. Experiments are done

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with the collaboration of the group member and with the attempt to understand the background of the member's culture.

For further information on the Gestalt approach to group counseling, we recommend *Beyond the Hot Seat Revisited: Gestalt Approaches to Groups* (Feder & Frew, 2008) and *Gestalt Group Therapy: A Practical Guide* (Feder, 2006).

Psychodrama

Key Concepts of Psychodrama **Psychodrama** is primarily an action approach to group counseling in which clients explore their problems through role playing, enacting situations using various dramatic devices to gain insight, discover their own creativity, and develop behavioral skills. The scenes are played as if they were occurring in the here and now, even though they might have their origins in a past event or an anticipated situation. Using psychodrama, the client acts out or dramatizes past, present, or anticipated life situations and roles to gain deeper understanding, to explore feelings and achieve emotional release, and to develop new ways of coping with problems. Significant events are enacted to help the members of the group get in contact with unrecognized and unexpressed feelings, to provide a channel for the full expression of these feelings and attitudes, and to broaden their role repertoire.

A main concept of psychodrama involves encouraging members to work in the present moment, as is also true of Gestalt therapy. At times group members will *talk about* situations in the past or the future to distance and defend themselves against experiencing their feelings. By re-creating those difficult situations as if they were happening in the present moment, the actual encounter is brought into consciousness. When the protagonist (group member who is the focus of the work) slips into narrating, or begins to *talk about* a problem, the psychodrama leader steers the protagonist into action by saying, "Don't tell me *about* it, show me what happened, as if it's happening now." By bringing a problem into the here and now, the participants move away from abstract and intellectual discussions. A key concept of psychodrama is that reliving and reexperiencing a scene from the past gives the participants both the opportunity to examine how that event affected them at the time it occurred and a chance to deal differently with the event *now.* By replaying a past event "as if" it were happening in the present, the individual is able to assign new meaning to it. Through this process, the person can work through unfinished business and reframe that earlier situation.

Therapeutic Goals of Psychodrama Psychodrama aims at fostering creativity in the individual, the group, and ultimately in the culture as a whole. Key goals of psychodrama are to facilitate the release of pent-up feelings, to provide insight, and to help group members develop new and more effective behaviors. In a group situation emotions tend to be released, which is the **catharsis** that often accompanies the experiential aspect of therapy. Catharsis is a natural part of the psychodramatic process, but it is not in itself a goal. Simply rediscovering buried emotions will not bring about healing; these feelings must be worked through for integration to occur. For those who have lost awareness of the roots of their feelings,

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emotional release may lead to insight, or to an increased awareness of a problem situation. Other goals of psychodrama include encouraging participants to live in the present and to behave in more spontaneous ways. A main aim is to open up unexplored possibilities for solving conflicts and for living more creatively.

Therapeutic Relationship The underlying philosophy of psychodrama is consistent with many of the premises of existential therapy, person-centered therapy, and Gestalt therapy, all of which emphasize understanding and respecting the group member's experience and the importance of the therapeutic relationship as a healing factor. Although practitioners who employ psychodramatic methods assume an active and directive role, these techniques are most effective when the group counselor adopts a person-centered spirit. Practitioners who are authentic, able to make connections with members, able to be psychologically present, demonstrate empathy, and exhibit a high level of respect and positive regard for the members of their groups are most effectively able to implement a range of psychodrama techniques.

Techniques The active techniques, such as role playing, are useful for many different kinds of groups. These methods enable group members to directly experience their conflicts to a much greater degree than is the case when members *talk about* themselves in a storytelling manner. This direct experiencing tends to bring emotions to the surface. Oftentimes one member's deeply emotional work will trigger emotional reactions in other members. The work of members can be linked to common themes emerging in a session. Participants in a psychodrama can be encouraged to reflect on how their beliefs and early decisions may be contributing to some of the emotional reactions they reexperienced in the psychodrama. The techniques of psychodrama encourage people to express themselves more fully, explore both intrapsychic conflicts and interpersonal problems, get constructive feedback on how they come across to others, reduce feelings of isolation, and experiment with novel ways of approaching significant others in their lives.

Psychodrama uses a number of specific techniques designed to intensify feelings, clarify implicit beliefs, increase self-awareness, and practice new behaviors. One of the most powerful tools of psychodrama is **role reversal**, which involves the group member taking on the part of another person. Through role reversal, people are able to get outside of their own frame of reference and enact a side of themselves they would rarely show to others. In addition, by reversing roles with a significant person, the person is able to formulate significant emotional and cognitive insights into his or her part in a relationship. This technique also creates empathy for the position of the other person.

A technique that has many applications for various kinds of groups is the technique of **future projection**, which is designed to help group members express and clarify concerns they have about the future. In future projection, an anticipated event is brought into the present moment and acted out. These concerns may include wishes and hopes, dreaded fears of tomorrow, and goals that provide some direction to life. Members create a future time and place with selected people, bring this event into the present, and get a new perspective on a problem.

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Members may act out either a version of the way they hope a given situation will ideally unfold or their version of the most horrible outcome. When participants in psychodrama enact anticipated events as though they were taking place in the here and now, they achieve an increased awareness of their available options. Rehearsals for future encounters, coupled with constructive and specific feedback, can be of real value to those members who want to develop more effective ways of relating to significant people in their lives.

Caution should be used when employing psychodrama techniques. Competent practitioners who use a psychodrama framework for practice devote a great deal of time to developing their skills, and they will have undergone a training program under the supervision of an experienced clinician. Psychodrama works best with clinicians who are well grounded in professional judgment and open to drawing methods from various approaches. It is important to remember that practitioners can use certain aspects or techniques of psychodrama without employing a complete enactment.

Multicultural Applications of Psychodrama If group members are uncomfortable in talking about deeply personal matters, let alone displaying their emotions in front of others, some psychodrama techniques are most likely not appropriate. Many of these techniques can be adapted to a problem-solving approach that makes use of cognitive and behavioral principles. It is possible to combine both didactic and experiential methods in structured groups with members who are culturally diverse. Role-playing techniques can be productively adapted to structured situations dealing with trying on a new set of specific behaviors. This makes for a possible integration of certain psychodrama techniques with many of the cognitive behavioral approaches.

An easy-to-read treatment of psychodrama is *Acting-In: Practical Applications of Psychodramatic Methods* (Blatner, 1996); we also recommend *Foundations of Psychodrama: History, Theory, and Practice* (Blatner, 2000).

LO7

Cognitive Behavioral Approaches

Some of the main cognitive behavioral group approaches include behavior therapy, cognitive therapy, rational emotive behavior therapy, and reality therapy. Although the cognitive behavioral approaches are quite diverse, they do share these attributes:

1. A collaborative relationship between the group member and the therapist
2. The premise that psychological distress is largely a function of disturbances in cognitive processes
3. A focus on changing cognitions to produce desired changes in affect and behavior
4. A present-focused model
5. A generally time-limited and educational treatment focusing on specific and structured target problems
6. A model that relies on empirical validation of its concepts and techniques

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The cognitive behavioral approaches are based on a structured, psychoeducational model, and they tend to emphasize the role of homework, place responsibility on the group member to assume an active role both during and outside of the group sessions, and draw from a variety of cognitive and behavioral techniques to bring about change.

A basic assumption underlying the cognitive behavioral approaches is that most problematic behaviors, cognitions, and emotions have been learned and can be modified by new learning. Members of a group are involved in a teaching and learning process and are taught how to develop a new perspective on ways of learning. They are encouraged to try out more effective behaviors, cognitions, and emotions. Problems may arise due to a skills deficit—adaptive behaviors or cognitive strategies that have not been learned—and group members can acquire coping skills by participating in this educational experience. An example would be providing social effectiveness skills to people with social phobias who are interested in confronting their fears.

A strength of the cognitive behavioral approaches is the wide range of techniques that participants can use to specify their goals and to develop the skills needed to achieve these goals. The specificity of the cognitive behavioral approaches helps group members translate fuzzy goals into concrete plans of action, which enable the members to keep these plans clearly in focus.

Cognitive behavioral therapy (CBT) has many applications to a variety of counseling groups in many different settings. Cognitive behavioral group therapy is effective for treating a wide range of emotional and behavioral problems. CBT in groups has been demonstrated to have beneficial results for specific problems such as anxiety, depression, phobia, obesity, eating disorders, dual diagnoses, and dissociative disorders. Some factors that make CBT effective in working with diverse clients include tailoring treatment to each individual, addressing the role of the external environment, the active and directive role of the therapist, the emphasis on education, relying on empirical evidence, the focus on present behavior, and the brevity of the approach.

Behavior Therapy

Key Concepts of Behavior Therapy The cornerstone of **behavior therapy** is the identification of specific goals at the outset of the therapeutic process, which serves as a way to monitor and measure the progress of group members. Because therapy begins with an assessment of baseline data, the degree of progress can be evaluated by comparing group members' behavior on a given dimension at any point in a group with the baseline data. Participants in a group are frequently challenged to answer the question, "Is what we are doing in here helping you make the changes you desire?" With this information, members are in the best position to determine the degree to which their personal goals are being achieved.

Behavior therapy as applied to group work is a systematic approach that begins with a comprehensive assessment of the individual to determine the present level of functioning as a prelude to setting therapeutic goals. After the group member establishes clear and specific behavioral goals, the therapist typically suggests strategies that are most appropriate for meeting these stated goals. Evaluation is

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used to determine how well the procedures and techniques are working. Empirically supported techniques are selected to deal with specific problems because this approach is grounded in evidence-based practice. **Evidence-based practice** is best conceived of in a broad way and includes clinician expertise, the best available research, and evaluating the client's characteristics, culture, and preferences. Even in behavior therapy, the therapeutic relationship is of central importance and critical to outcome.

Therapeutic Goals of Behavior Therapy The general goals of behavior therapy are to increase personal choice and to create new conditions for learning. An aim is to eliminate maladaptive behaviors and to replace them with more constructive patterns. The client and therapist collaboratively specify treatment goals in concrete, measurable, and objective terms. Goals must be clear, concrete, understood, and agreed on by the members and the group leader. Behavior therapists and group members alter goals throughout the therapeutic process as needed.

Therapeutic Relationship A good working relationship is an essential precondition for effective group therapy to occur. The skilled group counselor can conceptualize problems behaviorally, use a range of specific behavioral techniques, and make use of the therapeutic relationship in bringing about change. The group counselor's role is to teach concrete skills through the provision of instructions, modeling, and performance feedback. Leaders tend to be active and directive and to function as consultants and problem solvers. Group members must be actively involved in the therapeutic process from beginning to end, and they are expected to cooperate in carrying out therapeutic activities, both in the sessions and outside of therapy.

Techniques Assessment is done at the outset of group treatment to determine a treatment plan. The leader follows the progress of group members through the ongoing collection of data before, during, and after all interventions. Such an approach provides both the group leader and the members with continuous feedback about therapeutic progress. Behavioral interventions are individually tailored to specific problems experienced by different group members.

Any technique that can be demonstrated to change behavior may be incorporated in a treatment plan. Techniques such as relaxation methods, role playing, behavioral rehearsal, coaching, guided practice, modeling, giving feedback, mindfulness skills, cognitive restructuring, systematic desensitization, in vivo desensitization, flooding, problem solving, and homework assignments can be included in any group counselor's practice, regardless of theoretical orientation. Some of these techniques can be demonstrated by both group leaders and other members in the group. For example, both the leader and members can provide modeling, such as dealing with others respectfully, taking risks by moving outside their comfort zone, and expressing empathy for others. During role-playing situations, both members and the leader can function as coaches by suggesting alternative and more productive dialogue. Both the leader and members offer beneficial feedback to others.

Behavioral treatment interventions are individually tailored to specific problems experienced by different group members. The behavioral group practitioner uses strategies that have research support for use with a particular kind of problem.

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These evidence-based strategies are used to promote generalization and maintenance of behavior change. Many of these research-based behavioral interventions can be fruitfully integrated into other therapeutic approaches.

Multicultural Applications of Behavior Therapy Behavioral approaches can be appropriately integrated into counseling with culturally diverse client populations when culture-specific procedures are developed. In designing a change program for clients from diverse cultures, effective practitioners conduct a functional analysis of the problem situation. This assessment includes the cultural context in which the problem behavior occurs, the consequences both to the client and to the client's sociocultural environment, the resources within the environment that can facilitate change, and the impact that change may have on others in the client's surroundings. The approach emphasizes teaching group members about the therapeutic process and stresses changing specific behaviors. By developing their problem-solving skills, clients learn concrete methods for dealing with practical problems within their cultural framework. Behavioral group practitioners typically spend time preparing members to participate in a group experience. The group process is demystified and norms are made clear. This approach is likely to appeal to clients who have doubts about the value of a group experience but are interested in learning practical ways of coping with immediate problems.

For further discussion of behavioral group therapy, we recommend *Cognitive-Behavioral Therapy in Groups* (Bieling, McCabe, & Antony, 2006), which has ideas on how to structure and lead cognitive behavioral groups and describes a wide variety of cognitive and behavioral strategies useful in groups.

Cognitive Therapy

Key Concepts of Cognitive Therapy According to **cognitive therapy (CT)**, psychological problems stem from commonplace processes such as faulty thinking, making incorrect inferences on the basis of inadequate or incorrect information, and failing to distinguish between fantasy and reality. Cognitive therapy assumes that people are prone to learning erroneous, self-defeating thoughts but that they are capable of unlearning them. People perpetuate their difficulties through the beliefs they hold and their self-talk. By pinpointing these cognitive errors and correcting them, individuals can create a more fulfilling life.

Automatic thoughts are personalized notions that are triggered by particular stimuli that lead to emotional responses. For example, a group member's automatic thought might be: "I am stupid because I can't follow what others are saying in this group." These negative automatic thoughts come up quite spontaneously, and when this happens the group member feels anxious and embarrassed. The cognitive group therapist is interested in helping members identify their automatic thoughts and teaching them how to evaluate their thoughts in a structured way. The techniques are designed to identify and test members' misconceptions and faulty assumptions. Cognitive therapists are continuously active and deliberately interactive with the members of their groups. The leader strives to engage members' active participation and collaboration throughout all phases of the group experience.

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A cognitive behavioral orientation places emphasis on the group leader functioning as a teacher who encourages group members to learn skills to deal with the problems of living. The emphasis is on changing specific behaviors and developing problem-solving skills rather than expressing feelings. The cognitive therapist teaches group members how to identify inaccurate and dysfunctional cognitions through a process of evaluation. The group leader assists members in forming hypotheses and testing their assumptions, which is known as **collaborative empiricism**. The group leader rarely directly challenges the beliefs of members, but instead works collaboratively with members to examine the evidence for certain beliefs, test the validity of these beliefs, and look for more adaptive ways of thinking. Thus a group member might say, "Once people get know me, they will reject me." The validity of this assumption can be questioned and explored in the group context. Through a process of feedback, this member might discover that several people in the group are drawn to him and express a liking for him. This feedback can help him to evaluate the merits of his assumptions.

Through a collaborative effort, group members learn to discriminate between their own thoughts and events that occur in reality. Members learn the influence that cognition has on their feelings and behaviors and even on environmental events, particularly the distortions that they have acquired.

Cognitive restructuring plays a central role in the cognitive therapies. Group members sometimes engage in catastrophic thinking by dwelling on the most extreme negative aspects of a situation. The leader can assist members in detecting those times when members get stuck imagining the worst possible outcome of a situation by asking these questions: "What is the worst thing that could occur?" and "If this happens, what would make this such a negative outcome?" Group participants are able to make changes by listening to their self-talk, by learning a new internal dialogue, and by learning coping skills needed for behavioral changes. In the group context, members are taught to recognize, observe, and monitor their own thoughts and assumptions, especially their negative automatic thoughts. Once members discover certain beliefs that are not accurate, they are encouraged to try out a different set of beliefs and behaviors, both in the group and in daily life.

Therapeutic Goals of Cognitive Therapy The goal of cognitive behavior therapy is to change the way clients think by identifying their automatic thoughts and begin to introduce the idea of cognitive restructuring. Changes in beliefs and thought processes tend to result in changes in the way people feel and how they behave. Members learn practical ways to identify their underlying faulty beliefs, to critically evaluate these beliefs, and to replace them with constructive beliefs.

Therapeutic Relationship Group leaders combine empathy and sensitivity with technical competence in establishing their relationship with members. A therapeutic alliance is a necessary first step in cognitive group therapy, especially in counseling difficult-to-reach group members. Group leaders must have a cognitive conceptualization of cases, be creative and active, be able to engage clients through a process of Socratic questioning, and be knowledgeable and skilled in the use of cognitive and behavioral strategies. Cognitive practitioners are continuously

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active and deliberately interactive with group members, helping them frame their conclusions in the form of testable hypotheses.

Techniques Cognitive therapy is present-centered, psychoeducational, and time-limited. Cognitive therapy in groups emphasizes a **Socratic dialogue** and helping group members discover their misconceptions for themselves. After group members have gained insight into how their unrealistically negative thoughts are affecting them, they are trained to test these inaccurate thoughts against reality by examining and weighing the evidence for and against them. This process involves empirically testing their beliefs by actively participating in a variety of methods, such as engaging in a process of guided discovery (or a Socratic dialogue) with the therapist, critically evaluating the bases for their beliefs, carrying out homework assignments, gathering data on assumptions they make, keeping a record of activities, and forming alternative interpretations. Through this process of **guided discovery**, the group leader functions as a catalyst and guide who helps the members understand the connection between their thinking and the ways they feel and act. Through the use of questioning the members acquire new information and different ways of thinking, acting, and feeling. The cognitive strategies of Socratic questioning and guided discovery are central to cognitive therapy, and they are frequently employed in a cognitively oriented groups.

The leader teaches group members how to be their own therapist. This includes educating participants in a group about the nature and course of their problems, about how cognitive therapy works, and how their thinking influences their emotions and behaviors. Cognitive behavioral practitioners function as teachers; group members acquire a wide range of skills to use in dealing with the problems of living. This educational focus appeals to many clients who are interested in learning practical and effective methods of bringing about change. Homework is often used in cognitive therapy, which is tailored to the member's specific problems and arises out of the collaborative therapeutic relationship. Homework is generally presented as an experiment, and group members are encouraged to create their own self-help assignments as a way to keep working on issues addressed in their group sessions. The educative process includes providing clients with information about their presenting problems and about **relapse prevention**, which consists of procedures for dealing with the inevitable setbacks clients are likely to experience as they apply what they are learning to daily life.

Multicultural Applications of Cognitive Therapy Cognitive therapy tends to be culturally sensitive because it uses the individual's belief system, or worldview, as part of the method of self-change. The therapist's attention is on the degree to which these beliefs are adaptive for the client. Cognitive therapists do not impose their beliefs on group members; rather, they help members assess whether a given belief fosters emotional well-being. The collaborative nature of CT offers members of a group the structure many people want, yet the group counselor still strives to enlist clients' active participation in the therapeutic process.

Some useful resources for learning more about cognitive therapy include *Cognitive Behavior Therapy: Beyond the Basics* (Beck, 2011) and *Cognitive Therapy for Challenging Problems: What to Do When the Basics Don't Work* (Beck, 2005).

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Rational Emotive Behavior Therapy

Key Concepts of Rational Emotive Behavior Therapy From the perspective of **rational emotive behavior therapy (REBT)**, our problems are caused by our perceptions of life situations and our thoughts, not by the situations themselves, not by others, and not by past events. It is our responsibility to recognize and change self-defeating thinking that leads to emotional and behavioral disorders. REBT also holds that people tend to incorporate these dysfunctional beliefs from external sources and then continue to indoctrinate themselves with this faulty thinking. To overcome irrational thinking, therapists use active and directive therapy procedures, including teaching, suggestion, and giving homework. REBT in groups emphasizes education, with the group leader functioning as a teacher and the group members as learners. Although REBT is didactic and directive, its goal is to get people to think, feel, and act for themselves. In a REBT group, members are consistently encouraged to do what is necessary to make long-lasting and substantive changes.

REBT group practitioners employ a directive role in encouraging members to commit themselves to practicing in everyday situations what they are learning in the group sessions. They view what goes on during the group as important, but they realize that the hard work between sessions and after therapy is terminated is even more crucial.

Therapeutic Goals of Rational Emotive Behavior Therapy The goals of REBT are to eliminate a self-defeating outlook on life, to reduce unhealthy emotional responses, and to acquire a more rational and tolerant philosophy. REBT offers group members practical ways to identify their underlying faulty beliefs, to critically evaluate these beliefs, and to replace them with constructive beliefs. Participants in a group learn how to substitute preferences for demands.

Therapeutic Relationship REBT practitioners strive to unconditionally accept the members of their groups and to teach them to unconditionally accept others and themselves. Group leaders do not blame or condemn members; rather, they teach members how to avoid rating and condemning themselves. REBT practitioners accept their clients as imperfect beings who can be helped using a variety of techniques.

Techniques REBT utilizes a wide range of cognitive, emotive, and behavioral methods with most group members. This approach blends techniques to change members' patterns of thinking, feeling, and acting. Techniques are designed to induce clients to critically examine their present beliefs and behavior. REBT focuses on specific techniques for changing a group member's self-defeating thoughts in concrete situations. In addition to modifying beliefs, REBT helps clients see how their beliefs influence what they feel and what they do.

One cognitive technique used in REBT groups is teaching members **coping self-statements**. Group members are taught how faulty beliefs can be countered by rational, coping self-statements. They are expected to monitor their manner of speaking by writing down and analyzing the quality of their language. For example, a member might tell herself: "I *must* perform well, which means being perfect.

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People will give me approval only when I'm perfect, and I very much need this approval from others to feel worthwhile." By becoming aware of the demanding quality of her internal and external speech, she can learn how what she tells herself is setting her up for failure. She can replace these self-defeating statements with coping statements: "I can still accept myself even though I am imperfect."

In REBT groups, **faulty thinking** is confronted. For example, when Jeffrey says, "I am a failure as a father, and I am sure that my children will never forgive me for the mistakes I have made," Jeffrey could be invited to go to each person and list one of the ways he has failed his children. The leader can ask Jeffrey to tell a few members in the group how he came to the conclusion that he is a failure and then acknowledge that he has made mistakes. Jeffrey might then critically evaluate the conclusion that he believes he is a total failure because he has made mistakes. Without knowing some of Jeffrey's history, it is difficult to assess the accuracy of his conclusion that he failed his children and that they would never forgive him. Exploring some of the details regarding the ways Jeffrey believes he has failed his children could help him move toward being more self-accepting in spite of his imperfections.

Multicultural Applications of Rational Emotive Behavior Therapy Some factors that make REBT effective in working with diverse client populations include tailoring treatment to each individual, the focus on present behavior, and the brevity of the approach. REBT group practitioners function as teachers, and this educational focus appeals to many group members who are interested in learning practical and effective methods of bringing about change. REBT stresses the relationship of individuals to the family, community, and other systems. This orientation is consistent with valuing diversity and the interdependence of being an individual and a productive member of the community.

For a comprehensive coverage of group REBT in different contexts and with different client populations, see *Rational Emotive Behavior Therapy* (Ellis & Ellis, 2011).

Choice Theory/Reality Therapy

Key Concepts of Choice Theory/Reality Therapy Choice theory rests on the assumption that humans are internally motivated and behave to control the world around them according to some purpose within them. **Choice theory**, which is the underlying philosophy of the practice of reality therapy, provides a framework that explains the why and how of human behavior. **Reality therapy** is based on the assumption that human beings are motivated to change (1) when they determine that their current behavior is not getting them what they want and (2) when they believe they can choose other behaviors that will get them closer to what they want.

Reality therapy group leaders expect group members to make an assessment of their current behavior to determine if what they are doing and thinking is getting them what they want from life. Group members are encouraged to explore their perceptions, share their wants, and make a commitment to counseling. Because clients can directly control their acting and thinking functions more than they can

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control what they are feeling, their actions become the focus of work in the group. The group members explore the direction in which their behavior is taking them and evaluate what they are doing. They then create a plan of action to make the changes they want. A key concept of reality therapy and choice theory is that no matter how dire our circumstances may be, we always have a choice. An emphasis of reality therapy is on assuming personal responsibility and on dealing with the present.

Therapeutic Goals of Reality Therapy The overall goal of this approach is to help people find better ways to meet their needs for survival, love and belonging, power, freedom, and fun. Changes in behavior tend to result in the satisfaction of basic needs. Much of the work in a group centers around members exploring the degree to which they are meeting their needs and determining better ways of meeting their needs and the needs of others in their lives.

Therapeutic Relationship Group members need to know that the leader cares enough about them to accept them and to help them fulfill their needs in the real world. Both *involvement with* and *concern for* group members are demonstrated by the leader throughout the life of a group. Once this therapeutic relationship has been established, the leader challenges members with the reality and consequences of their actions. The group leader avoids criticism, refuses to accept members' excuses for not following through with agreed-upon plans, and does not easily give up on members. The leader assists members in the continual process of evaluating the effectiveness and appropriateness of their current behavior.

Techniques The practice of a reality therapy group can best be conceptualized as the cycle of counseling, which consists of two major components: (1) the counseling environment and (2) specific procedures that lead to change in behavior. Reality therapy is active, directive, and didactic. The group leader assists members in making plans to change those behaviors that they determine are not working for them. Skillful questioning and various techniques are employed to help members make this self-evaluation.

Reality therapy is active, directive, and didactic.

Some of the specific procedures in the practice of reality therapy have been developed by Robert Wubbolding (2011). These procedures are summarized in the **WDEP model**, which refers to the following clusters of strategies:

W = wants: exploring wants, needs, and perceptions.
D = direction and doing: focusing on what clients are doing and the direction this is taking them.
E = evaluation: challenging clients to make an evaluation of their total behavior.
P = planning and commitment: assisting clients in formulating realistic plans and making a commitment to carry them out.

In a group, members explore what they want, what they have, and what they are not getting. The cornerstone of a reality therapy group is assisting members in making a self-evaluation of their current behavior. This self-assessment provides a basis for making specific changes that will enable the members to reduce their frustrations. Useful questions can help them pinpoint what they want: "What

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kind of person do you wish you were?" "What would you be doing if you were living the way you wished?" "Is this choice to your best short-term and long-term advantage, and is it consistent with your values?" The purpose of such questions is to help clients move from a sense of external control to a sense of internal control. This line of questioning sets the stage for the application of other procedures in reality therapy.

Creating an **action plan** is an essential part of the process of a reality therapy group. Carrying out plans enables people to gain effective control over their lives. Therapy is best directed toward providing members with new information and helping them discover more effective ways of getting what they want. A large portion of the therapy time consists of making plans and then checking to determine how these plans are working. In a group context, members learn how to plan realistically and responsibly through contact with both the other members and the leader. The members are encouraged to experiment with new behaviors, to try out different ways of attaining their goals, and to carry out an action program. Effective plans are modest in the beginning and specify what is to be done, when it will be done, and how often. In short, plans are meant to encourage group members to translate their talk and intentions into actions.

Multicultural Applications of Reality Therapy Reality therapy assumes that the basic needs (survival, belonging, power, freedom, and fun) are universal, yet the ways in which these needs are expressed depend largely on the cultural context. When working with culturally diverse clients, it is essential that group leaders allow latitude for a diverse range of acceptable behaviors to satisfy these needs. Reality therapists demonstrate their respect for the cultural values of their clients by helping them explore how satisfying their current behavior is, both to themselves and to others. After group members make a self-assessment, they identify those areas of living that are *not* working for them. Members are then in a position to formulate specific and realistic plans that are consistent with their cultural values.

For more on reality therapy, we recommend *Reality Therapy: Theories of Psychotherapy Series* (Wubbolding, 2011).

LO8

Postmodern Approaches

The postmodern approaches (solution-focused brief therapy, narrative therapy, motivational interviewing, and feminist therapy) challenge many of the assumptions of traditional therapies. Postmodernism is based on the premise that there is no single truth. Postmodern perspectives are marked by acceptance of plurality and the notion that individuals create their own reality. The basic premise is that people are resourceful, competent, healthy, resilient, and have the capacity to discover solutions that can change the direction of their lives. Individuals are the experts on their own lives. The postmodern approaches have in common the basic assumption that we generate stories to make sense of ourselves and our world.

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Both solution-focused brief therapy and narrative therapy cast the role of a therapist in a different light from traditionally oriented therapists who view themselves as experts in assessment and treatment. Solution-focused therapists and narrative therapists adopt a "not-knowing" position and believe the members of their groups are the true experts on their own lives. Solution-focused brief therapy moves quickly from "problem-talk" to "solution-talk" and focuses on keeping therapy simple and brief. By talking about the exceptions to a problem, group members are able to conquer what seem to be major problems. In narrative therapy, people are empowered by learning how to separate themselves from their problems. By emphasizing positive dimensions of human experience, clients become actively involved in resolving their problems. Instead of aiming to make change happen, the practitioners who operate from these two approaches attempt to create an atmosphere of understanding and acceptance that enables individuals to utilize their own resources to make constructive changes.

Motivational interviewing is a humanistic, client-centered, psychosocial, and modestly directive counseling approach that was developed by William R. Miller and Stephen Rollnick in the early 1980s. Motivational interviewing was initially designed as a brief intervention for problem drinking, but it has been effectively applied to a wide range of problems. Practitioners stress client self-responsibility and promote working cooperatively with the client to generate alternative solutions to behavioral problems.

From the beginning, feminist therapy groups actively worked to establish shelters for battered women, rape crisis centers, and women's health and reproductive health centers. Building community, providing authentic mutual empathic relationships, creating a sense of social awareness, and emphasizing social change are all significant strengths of this approach to group work. Feminist theory encompasses a broader perspective than do most theories that value the marginalized voices of both men and women. Members in a group that is structured by feminist principles can expect more than simple problem-solving strategies. They need to be prepared for major shifts in their way of viewing the world around them, changes in the way in which they perceive themselves, and transformed interpersonal relationships.

Solution-Focused Brief Therapy

Key Concepts of Solution-Focused Brief Therapy A central concept of a **solution-focused brief therapy (SFBT)** group includes a movement from talking about problems to talking about and creating solutions. Members learn to pay attention to what is working, and then do more of this. Change is constant and inevitable, and small changes pave the way for larger changes. Little attention is paid to pathology or to giving people a diagnostic label. There are exceptions to every problem, and by talking about these exceptions, group participants are able to conquer what seem to be major problems in a brief period of time.

Therapeutic Goals of Solution-Focused Brief Therapy The solution-focused model emphasizes the role of participants establishing their own goals and preferences. Much of what the group process is about involves members thinking about

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their future and what they want to be different in their lives. Leaders concentrate on clear, specific, observable, small, realistic, achievable changes that may lead to additional positive outcomes. Because success tends to build upon itself, modest goals are viewed as the beginning of change.

Therapeutic Relationship SFBT is a collaborative venture; the group therapist strives to carry out therapy *with* an individual, rather than doing therapy *on* an individual. Empathy and the collaborative partnership in the therapeutic process are seen as more important than assessment or technique. Instead of aiming to *make* change happen, the leader attempts to create an atmosphere of understanding and acceptance that allows individuals to tap their resources for making constructive changes. Group members are the primary interpreters of their own experiences. Together the group members and the leader establish clear, specific, realistic, and personally meaningful goals that will guide the process of a group. This spirit of collaboration opens up a range of possibilities for present and future change.

Solution-focused group counselors believe that the way problems and solutions are talked about makes a difference. The language used signals how problems are conceptualized. The concepts of care, interest, respectful curiosity, openness, empathy, contact, and even fascination are seen as relational necessities. Group leaders create a climate of mutual respect, dialogue, inquiry, and affirmation in which the members are free to create and coauthor their evolving stories. Because solution-focused counseling is designed to be brief, the leader has the task of keeping group members on a *solution track* rather than a *problem track*. If members concentrate on talking about their problems, it is difficult for them to move in a positive direction.

Techniques Solution-focused therapists use a range of techniques including pretherapy change, exception questions, the miracle question, scaling questions, homework, and summary feedback. Some group practitioners ask the member to externalize the problem and focus on strengths or unused resources. Others challenge group members to discover solutions that might work. Techniques focus on the future and how best to solve problems rather than on understanding the cause of problems.

Solution-focused brief therapists often ask clients at the first session, "What have you done since you called for an appointment that has made a difference in your problem?" Asking about **pretherapy change** tends to encourage clients to rely less on the therapist and more on their own resources to reach their goals.

Questioning is a main intervention. Solution-focused group leaders use questions as a way to better understand a group member's experience rather than simply to gather information. Group leaders do not raise questions to which they think they know the answer. Questions are asked from a position of respect, genuine curiosity, sincere interest, and openness. Instead of presenting members with a barrage of closed, interrogation-style questions, open-ended questions can enhance solutions by providing space for members to be heard and to reflect on future possibilities. Asking questions enables members to describe things in their own words: "Tell me more about your concerns."; "What do you think needs to

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happen to make things a little better at work?"; "Who will be the first to notice when things get better, and how might they react?"

Exception questions direct group members to those times in their lives when their problems did not exist. Exploring exceptions offers group members opportunities for discovering resources, engaging strengths, and creating possible solutions. Solution-focused group counselors listen attentively for signs of previous solutions, exceptions, and goals. For example, Randy says: "I feel tired and *depressed* most of the time; I get angry at my kids over something they didn't do *almost* every night." Exception-finding questions include "When is this problem absent or less noticeable?"; "Has anything changed for the better since the last group session?" Once an exception is discovered, the leader can explore exception-related conditions and encourage a member to replicate these conditions: "What was different about yesterday when you felt less depressed?" "What will it take to keep depression at bay more often?" The intent of this solution-focused intervention is to guide Randy in a self-chosen direction based on what has worked previously. Randy can be asked what has to happen for these exceptions to his problems to occur more often.

A related intervention to exception questions involves changing what one is doing. For example, the leader might ask Chuck if he is willing to do something different the next time he feels worried and anxious. After hearing Chuck say that he worries most of the time and has tried many methods to stop without success, the leader might invite Chuck to schedule an intensive 10-minute worry session in the morning to give his concerns the undivided attention these worries deserve. If Chuck carries out this experiment, he is likely to discover that he has some control over his feelings by changing what he is doing in certain situations.

The **miracle question** allows members of a group to describe life without the problem. The miracle question is often presented as follows: " If a miracle happened and the problem you have disappeared overnight, how would you know it was solved, *and what would be different*?" Group members are then encouraged to enact "what would be different" in spite of perceived problems. This question involves a future focus that encourages group members to consider a different kind of life than one dominated by a particular problem. The miracle question focuses clients on searching for solutions and recognizing small improvements toward their goals.

Scaling questions require group members to specify improvement on a particular dimension on a scale of zero to 10. This technique enables clients to see progress being made in specific step and degrees. Solution-focused therapists use scaling questions when changes in human experiences are not easily observed, such as feelings, moods, or communication. For example, a group member reporting feelings of anxiety in social situations might be asked, "On a scale of zero to 10, with zero being how you felt when you first came to this group and 10 being how you feel the day after your miracle occurs and your problem is gone, how would you rate your anxiety right now?" Even if the group member has only moved away from zero to 1, she has improved. How did she do that? What does she need to do to move another number up the scale? Scaling questions enable clients to pay closer attention to what they are doing and how they can take steps that will lead to the changes they desire.

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Therapists may provide **summary feedback** in the form of genuine affirmations or pointing out particular strengths that clients have demonstrated. Solution-focused practitioners typically allow time in each group session for sharing feedback with one another. The leader gives members credit for the changes they are making through statements such as these: "How did you manage to make these improvements?" "How does your family treat you differently now compared to before?" "What have these improvements taught you about yourself?" "What is the biggest difference between the old you and the new you?" These questions provide encouragement for members to search within themselves for the credit they deserve for making changes. This kind of questioning and feedback assists members in carrying their learning outside of the group sessions into daily living.

Multicultural Applications of Solution-Focused Brief Therapy Solution-focused brief therapists learn from their clients about their experiential world rather than approaching clients with a preconceived notion about their worldview. The nonpathologizing stance taken by solution-focused practitioners moves away from dwelling on what is wrong with a person to emphasizing creative possibilities. Instead of aiming to make change happen, the SFBT practitioner attempts to create an atmosphere of understanding and acceptance that allows a diverse range of individuals to utilize their resources for making constructive changes. Murphy (2015) claims that the emphasis on strengths and resources in solution-focused counseling supports culturally competent services to individuals regardless of ethnicity and cultural background. Some specific aspects of solution-focused counseling that lend themselves to culturally competent practice include (a) treating each client as a unique individual, (b) collaborating on the goals of counseling, (c) tailoring services to each client, and (d) obtaining ongoing feedback from clients on the usefulness of interventions and adjusting them accordingly.

For a more comprehensive account of SFBT, we highly recommend *Solution-Focused Counseling in Schools* (Murphy, 2015) and *Solution-Focused Group Therapy: Ideas for Groups in Private Practice, Schools, Agencies and Treatment Programs* (Metcalf, 1998).

Narrative Therapy

Key Concepts of Narrative Therapy **Narrative therapy** is based partly on examining the stories that people tell and understanding the meaning of their stories. Each of these stories is true for the individual who is telling the story; there is no absolute reality. Some key concepts of narrative therapy include a discussion of how a problem has been disrupting, dominating, or discouraging the person. The therapist attempts to separate clients from their problems so that they do not adopt a fixed view of their identity. Group members are invited to view their stories from different perspectives and eventually to co-create an alternative life story. They are asked to find evidence to support a new view of themselves as being competent enough to escape the dominance of a problem and are encouraged to consider what kind of future they could expect if they were competent. In essence, group members reauthor their stories about themselves and their relationships.

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Therapeutic Goals of Narrative Therapy Narrative therapists invite group members to describe their experience in fresh language, which tends to open new vistas of what is possible. The heart of the therapeutic process from the perspective of narrative therapy involves identifying how societal standards and expectations are internalized by people in ways that constrain and narrow the kind of life they are capable of living. Narrative therapists collaborate with the members of their groups to help them experience a heightened sense of personal agency to act in the world. The member is asked to find evidence to support a new view of being competent enough to escape the dominance of a problem and is encouraged to consider what kind of future could be expected from the competent person who is emerging.

Therapeutic Relationship Therapy is a collaborative venture aimed at assisting group members to construct meaningful goals that will lead to a better future. Narrative therapists do not assume that they have special knowledge about the lives of clients. The members of a group are the primary interpreters of their own experiences. In the narrative approach, the therapist seeks to understand the lived experience of each of the group participants. Through a systematic process of careful listening, coupled with curious, persistent, and respectful questioning, the group leader works collaboratively with members to explore the impact of the problem and what they are doing to reduce its effects. Members then work with the therapist to co-construct enlivening alternative stories. For example, the group leader may explore with Leilani her lived experiences about her courage, her ability to be decisive, her willingness to take risks in the face of uncertainty, and her ability to deal directly with her fears. In this discussion, occasions when Leilani has demonstrated courage in the face of her fears are identified. These include making a decision to pursue a master's degree program in counseling, taking risks by leaving a job that was stifling to her, and her ability to pursue social action projects that are meaningful to her. The group experience provides opportunities for Leilani to create a changing identity and to recruit an audience who will provide support for her as she moves more fully into her preferred story.

Techniques Narrative therapy emphasizes the quality of the therapeutic relationship and the creative use of techniques within this relationship. Narrative therapy's most distinctive feature is captured by the statement, "The person is not the problem, the problem is the problem." Narrative therapists engage clients in **externalizing conversations** that are aimed at separating the problem from the person's identity. Members learn that they are not cemented to their problem-saturated stories and can develop alternative and more constructive stories.

"The person is not the problem, the problem is the problem."

Narrative therapists use questions as a way to generate experience rather than to gather information. The aim of questioning is to progressively discover or construct the client's experience so that the client has a sense of a preferred direction. Therapists ask questions from a not-knowing position, meaning that they do not pose questions to which they already know the answers. Through the process of asking questions, therapists provide clients with an opportunity to explore various dimensions of their life situations.

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As narrative therapists listen to clients' stories, they pay attention to details that give evidence of clients' competence in taking a stand against an oppressive problem. Problems are not viewed as pathological manifestations but as ordinary difficulties and challenges of life. In the practice of narrative therapy, there is no recipe, no set agenda, and no formula to follow that will ensure a desired outcome.

Multicultural Applications of Narrative Therapy Narrative therapists operate on the premise that problems are identified within social, cultural, political, and relational contexts rather than existing within individuals, which makes this approach especially relevant for counseling culturally diverse clients. Narrative practitioners are concerned with considering the specifications of gender, ethnicity, race, disability, sexual orientation, social class, and spirituality and religion as therapeutic issues. The sociopolitical conceptualization of problems offers understanding of cultural notions and practices that produce dominant and oppressive narratives. Practitioners deconstruct, or take apart, the cultural assumptions that are a part of a client's problem situation. As a part of the group experience, members come to understand how oppressive social practices affect them, which allows for the possibility of creating alternative stories.

For a further treatment of narrative therapy, see *Narrative Counseling in Schools: Powerful and Brief* (Winslade & Monk, 2007).

Motivational Interviewing

Key Concepts of Motivational Interviewing **Motivational interviewing (MI)** is rooted in the philosophy of person-centered therapy, but with a "twist." Unlike the nondirective and unstructured person-centered approach, MI is deliberately directive, yet it stays within the client's frame of reference. Group counselors can integrate the principles of motivational interviewing with a number of approaches, especially person-centered therapy, cognitive behavior therapy, choice theory/reality therapy, and solution-focused brief therapy. MI practitioners believe in the client's abilities, strengths, resources, and competencies. MI has been effective in working with a wide range of behavioral issues, including substance abuse, compulsive gambling, eating disorders, anxiety disorders, depression, suicidality, chronic disease management, and health-related practices.

Therapeutic Goals of Motivational Interviewing The major goals of motivational interviewing are to explore an individual's ambivalence, to minimize this ambivalence, and to build intrinsic motivation. MI is built on the premise that people who seek therapy are often ambivalent about change and that motivation tends to ebb and flow during the course of therapy. By understanding both sides of a member's ambivalence regarding change, group counselors begin to work with this ambivalence rather than working against the client's struggles. MI can be a significant factor in helping clients commit to the therapy process, which improves client involvement, adherence, and retention in cognitive behavioral and other action-oriented therapies.

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Techniques MI emphasizes being purposeful and getting to the point to guide group members toward positive change. MI group leaders encourage members to decide whether they want to make certain changes. If members decide to change, group leaders ask what kinds of changes will occur and when they will occur. The attitudes and interventions in MI are based on a person-centered philosophy and include using open-ended questions, employing reflective listening, creating a safe climate, affirming and supporting the client, expressing empathy, responding to resistance in a nonconfrontational manner, guiding a discussion of ambivalence, summarizing and linking at the end of sessions, and eliciting and reinforcing *change talk*.

Therapeutic Relationship Practitioners emphasize the relational context of therapy, known as the "MI spirit." When this MI spirit is applied to group counseling, the group leader establishes collaborative partnerships with members and draws on their ideas and resources rather than assuming a role as the expert. All choices ultimately rest with the members, not the leader. MI is a collaborative, conversational style of working that strengthens an individual's own motivation to change. As group members begin to show signs of readiness to change, they increasingly talk about change, express a desire to change, experiment with possible change actions between sessions, and envision a future for themselves after the desired changes have been made.

Multicultural Applications of Motivational Interviewing MI is a phenomenological approach in which the counselor attempts to see life from the client's perspective. This perspective makes this approach suitable for working with clients from diverse cultural backgrounds. In MI groups, members are not persuaded to adopt the counselor's advice, nor are they expected to operate within the framework of the leader's values. Instead, members are encouraged to consider a menu of alternative interventions or options that fit well in their own lives.

For a further treatment of motivational interviewing, see *Motivational Interviewing: Helping People Change* (Miller & Rollnick, 2013) and *Motivational Interviewing in Groups* (Wagner & Ingersoll, 2013).

Feminist Therapy

Key Concepts of Feminist Therapy **Feminist therapy** focuses on issues of diversity, the complexity of sexism, and the centrality of social context in understanding gender issues. Feminist therapists have challenged the male-oriented assumptions regarding what constitutes a mentally healthy individual. Practitioners with a feminist orientation emphasize that gender-role expectations profoundly influence our identity from birth onward. Thus group counseling has the task of bringing to one's awareness how gender-role socialization is deeply ingrained in adult personality.

A key concept of feminist therapy is the notion that societal gender-role messages influence how individuals view themselves and behave. Throughout therapy the impact of these socialization patterns are identified so that clients can critically evaluate and modify early messages pertaining to appropriate gender-role

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behavior. The practice of contemporary feminist therapy is based on the assumption that gender cannot be considered apart from other identity areas such as race, ethnicity, class, and sexual orientation. A key concept pertaining to understanding symptoms is that problematic symptoms can be viewed as coping or survival strategies rather than as evidence of pathology. Although individuals are not to blame for personal problems largely caused by dysfunctional social environments, they are responsible for working toward change.

The first step toward change is becoming aware of how society has influenced our beliefs and behaviors, especially with respect to views pertaining to gender roles. Group members have an opportunity to identify their biases pertaining to gender-role identity. This can be a subtle learning, but it can lead to significant changes in how we relate to one another. For example, in one of our groups a man consistently referred to "the girls" that worked for him in the office. Several women in the group let him know they thought his remarks were inappropriate because his employees were mature women. He replied that he did not see it as a "big deal" that he called the women "girls," but eventually he was able to understand the implications of his references.

In my (Cindy) private practice, I have worked with numerous women struggling with symptoms of postpartum depression and have found a feminist perspective to be extremely helpful in understanding this complex issue. Many of the women I worked with were trying to balance work and being a new mother with little or no support from their male partners. Even though they had loving husbands and partners, the inequities in the responsibilities of child care and housework were startling. These women were doing it all and wondering why they were feeling inadequate, unfulfilled, isolated, and depressed. When viewed from a feminist perspective, postpartum depression is identified as a systemic issue rather than an individual illness. Rather than medicating these women for depression, feminist therapists would look toward a reexamination of role expectations and the distribution of work in relation to child care and other household duties. The symptoms these female clients were experiencing made sense in the context of their relationships. Their duties had been drastically increased without a corresponding increase in the amount of support provided by their partners.

Therapeutic Goals of Feminist Therapy The major goal of feminist therapy is empowerment; members of a group strive for a sense of self-acceptance, self-confidence, self-esteem, joy, and self-actualization. Other therapy goals include enhancing the quality of interpersonal relationships, assisting both women and men to make decisions regarding role performances, and helping group members to come to an understanding of the influence of cultural, social, and political systems on their current situation. Emphasis is on balancing independence and interdependence, social change, and valuing and affirming diversity. Feminist therapists do not see the therapeutic relationship alone as being sufficient to produce change. Insight, introspection, and self-awareness are springboards to action. Both individual change and social transformation are basic goals of therapy. At the individual level, therapists work to help women

Both individual change and social transformation are basic goals of therapy.

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and men recognize, claim, and embrace their personal power. As a consciously political enterprise, another goal is social transformation.

Therapeutic Relationship Feminist therapists work in an egalitarian manner and use empowerment strategies that are tailored to each client (Brown, 2010; Evans, Kincade, & Seem, 2011). They aim to empower clients to live according to their own values and to rely on an internal (rather than external or societal) locus of control in determining what is right for them. Group leaders with a feminist orientation are concerned about power relations in the world in general. The group leader and members take active and equal roles, working together to determine goals that members will pursue in a group.

The group practitioner works to demystify the therapeutic process and to include each member as an active partner in the treatment process. Collaboration with the members of the group leads to a genuine partnership with the members. When members are not informed about the nature of the therapeutic process, they are denied the opportunity to actively participate in the group experience. When practitioners make decisions about a client *for* the client rather than *with* the client, they rob the client of power in the therapeutic relationship.

Feminist therapists share common ground with Adlerian therapists in their emphasis on social equality and social interest. Also in line with Adlerians, feminist therapists believe the therapeutic relationship should be a nonhierarchical, person-to-person relationship. Like person-centered therapists, feminist therapists convey their genuineness and strive for mutual empathy between client and therapist. A common denominator of both feminist and other postmodern approaches is the assumption that each member is an expert on his or her own life.

Techniques Feminist therapy does not prescribe any particular set of interventions; rather, feminist group therapists tailor interventions to members' strengths. Feminist practitioners have borrowed techniques from traditional approaches including the use of therapeutic contracts, homework, bibliotherapy, therapist self-disclosure, empowerment, role playing, cognitive restructuring, reframing, relabeling, and assertiveness training. They typically draw techniques from the cognitive behavioral approaches in a group. In addition, Enns (2004) describes several unique techniques that feminist therapists have developed such as gender-role analysis and intervention, power analysis and intervention, and social action. **Gender-role analysis** explores the impact of gender-role expectations on the individual's psychological well-being and draws upon this information to make decisions about modifying gender-role behaviors. **Power analysis** refers to methods aimed at helping individuals understand how unequal access to power and resources can influence personal realities. Together the group leader and members explore how inequities or institutional barriers often limit self-definition and well-being. **Social action** is a defining feature of feminist therapy. As clients become more grounded in their understanding of feminism, therapists may suggest that clients become involved in activities such as volunteering at a community mental health center, lobbying lawmakers, or providing community education about gender issues. Participating in such activities can empower clients and help them see the link between their personal experiences and the sociopolitical context in which they live.

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Multicultural Applications of Feminist Therapy Feminist therapy and multicultural perspectives have a great deal in common. The feminist perspective on power in relationships has application for understanding power inequities due to racial and cultural factors. The "personal is political" principle can be applied both to counseling women and counseling culturally diverse client groups. Neither feminist therapy nor multicultural perspectives focus exclusively on individual change. Instead, both approaches emphasize direct action for social change as a part of the role of therapists. Many of the social action and political strategies that call attention to oppressed groups have equal relevance for women and for other marginalized groups. Both feminist therapists and multicultural therapists have worked to establish policies that lessen the opportunities for discrimination of all types—gender, race, culture, sexual orientation, ability, religion, and age.

For a further treatment of feminist therapy, see *Introduction to Feminist Therapy: Strategies for Social and Individual Change* (Evans et al., 2011) and *Feminist Therapy* (Brown, 2010).

LO9

An Integrative Approach

An **integrative approach** to the practice of group counseling is based on concepts and techniques drawn from various theoretical approaches. One reason for the current trend toward an integrative approach to the helping process is the recognition that no single theory is comprehensive enough to account for the complexities of human behavior when the full range of client types and their specific problems are taken into consideration.

Most clinicians now acknowledge the limitations of basing their practice on a single theoretical system and are open to the value of integrating various therapeutic approaches. Those clinicians who are open to an integrative perspective may find that several theories play crucial roles in their personal approach. Each theory has its unique contributions and its own domain of expertise. By accepting that each theory has strengths and weaknesses and is, by definition, different from the others, practitioners have some basis to begin developing a counseling model that fits them.

We encourage you to remain open to the value inherent in each of the theories of counseling. All the theories have some unique contributions as well as some limitations. Study all the contemporary theories to determine which concepts and techniques you can incorporate into your approach to the practice of group work. You will need to have a basic knowledge of various theoretical systems and counseling techniques to work effectively with diverse client populations in various settings. Functioning exclusively within the parameters of one theory may not provide you with the therapeutic flexibility that you need to deal creatively with the complexities associated with the diversity found in many groups.

Each theory represents a different vantage point from which to look at human behavior, but no one theory has the total truth. Because there is no "correct" theoretical approach, it is well for you to search for an approach that fits who you are and to think in terms of working toward an integrated approach that addresses

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thinking, feeling, and behaving. To develop this kind of integration, you need to be thoroughly grounded in a number of theories, be open to the idea that these theories can be unified in some ways, and be willing to continually test your hypotheses to determine how well they are working.

For those of you who are beginning your counseling career, it is probably wise to select the primary theory closest to your basic beliefs. Learn that theory as thoroughly as you can, and at the same time be open to examining what other theories can offer. If you begin by working within the parameters of a particular theory, you will have an anchor point from which to construct your own counseling perspective. But do not think that simply because you adhere to one theory you can use the same techniques with all of your clients. Even if you adhere to a single theory, you will need to be flexible in the manner in which you apply the techniques that flow from this theory as you work with diverse members in your groups.

If you are currently a student-in-training, it is unrealistic to expect that you will already have an integrated and well-defined theoretical model. An integrative perspective is the product of a great deal of reading, study, supervision, clinical practice, research, and theorizing. With time and reflective study, you can develop a consistent conceptual framework to use as a basis for selecting from the multiple techniques available to you. Take time to think about your therapeutic style and its influence on the process and outcomes of your group, then answer these questions: Can you describe the key features of your style in clear terms? To what degree are you able to conceptualize what you are attempting to accomplish through the group process? Developing your personalized approach that guides your practice is a lifelong endeavor that will be refined with experience.

Points to Remember

Theories and Techniques of Group Counseling

Here are some key points to remember; many of the following chapters are built on these basic concepts.

- It is important to have a theoretical rationale to help you make sense of what occurs in a group. Take the time to understand several theoretical orientations, and then select concepts from each to form your own personal style of working.
- A general theoretical framework helps you make sense of the many facets of group process, provides you with a map that allows you to intervene in a creative and effective manner, and provides a basis for evaluating the results of your interventions.
- The psychodynamic models of group counseling include psychoanalytic and Adlerian approaches.
- The experiential and relationship-oriented approaches to group counseling include existential, person-centered, Gestalt therapy, and psychodrama.
- The cognitive behavioral models include behavior therapy, cognitive therapy, rational emotive behavior therapy, and reality therapy.
- The postmodern perspectives include solution-focused brief therapy, narrative therapy, motivational interviewing, and feminist therapy.
- An integrative approach incorporates the thinking, feeling, and doing dimensions of human behavior and offers a number of advantages over subscribing to a single theoretical framework.

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Exercises

Questions for Discussion

1. In a small group, explore ways that you might have a primary theory and also draw from a couple of other theories to broaden your approach. At this point, what would be your primary theory, and what are your reasons for selecting it? What aspects from other models would you want to incorporate into your approach as a group counselor?
2. What are the advantages of practicing within a single theoretical perspective? What are some disadvantages? Do you see value in developing an integrative stance that draws on concepts and techniques from diverse theoretical perspectives? What are the potential difficulties when integrating elements from different theoretical models?
3. In a small group, discuss some aspects of your personal approach to counseling practice. The questions below may help you clarify your theoretical stance.
   a. What is your worldview, and how does it influence the way you make sense of your world, the people in it, and the interactions you have with others?
   b. In what ways are you willing to modify your approach to better serve diverse populations?
   c. What are your thoughts about how change happens?
   d. What do you believe about how people heal? In what context do people heal? What must be present in order for people to heal? What methods work to help people heal?
   e. What was your own counseling experience like for you? What was helpful or not helpful for you?
   f. What role or importance do you place on the relationship between client and counselor?
   g. What are the roles of the group member and the group counselor from your perspective?
   h. How do you define problems, problem situations, and problem behaviors?
   i. How do you decide what techniques to use in a group?

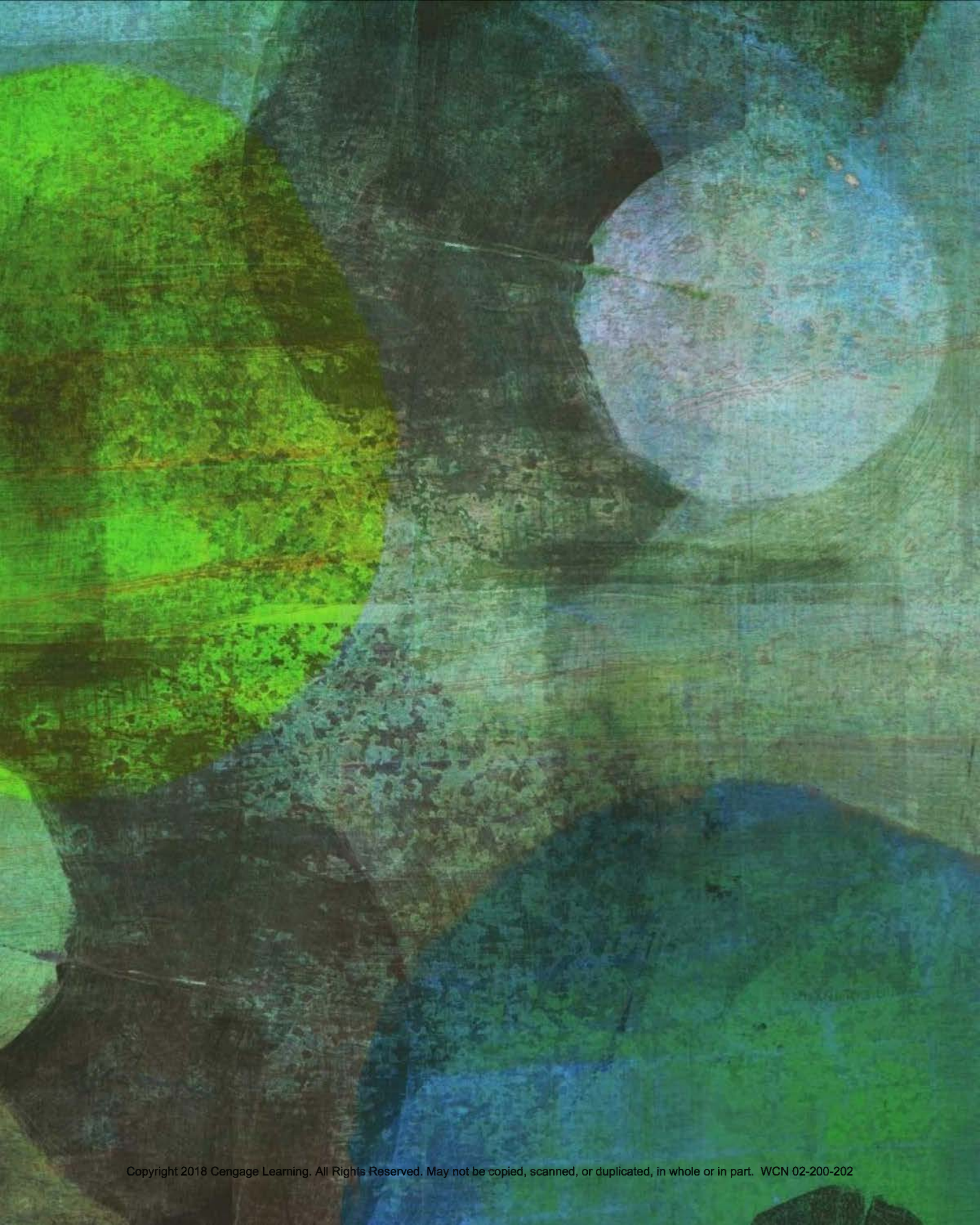
Guide to Groups in Action: Evolution and Challenges DVD and Workbook

If you are not seeing the DVD (*Groups in Action*) in class or working with this program, you can skip the exercises pertaining to this group program. If you are viewing the DVD, here are some suggestions for making the best use of this chapter along with Part One of the program, "Theories and Techniques of Group Counseling." This third and final part of the DVD program is based on a brief lecture by Gerald Corey that complements Chapter 4 of this textbook. In addition, the main points for each of the general theory areas discussed in this chapter are summarized in the workbook, along with questions for discussion that can provide the basis for small group activities in the classroom.

MindTap for Counseling

Additional resources can be found on CengageBrain.com and by logging into the MindTap course created by your professor. There you will find a variety of study tools and useful resources that include quizzes, videos, interactive exercises, and more.

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